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NIHR Health Technology Assessment Programme

Scheme	Commissioned
Funding Opportunity	2024/204 HTA Commissioned - Therapies for women, children and others who experience domestic abuse

SUMMARY INFORMATION

Application Title	A-PATH: Adapting Psychological Advocacy Towards Healing for older women affected by domestic abuse
Contracting Organisation	University College London
Contracting Organisation Department	Division of Psychiatry
Start Date	01/01/2026
End Date	31/12/2028
Duration (months)	36
Total Cost to NIHR	£335,383.93
NHS Support Costs	£0.00
NHS Excess Treatment Costs	£0.00
Non-NHS Excess Treatment Costs	£0.00
Has this application been previously submitted to this or any other funding body?	No

LEAD APPLICANT & RESEARCH TEAM

Lead Applicant

Full Name	Dr Vasiliki Orgeta
Organisation	University College London
Department	Division of Psychiatry
Proposed Role	Chief Co-lead investigator; project management, overseeing the development of the intervention and feasibility trial; trials and psychology lead; overseeing recruitment; results and dissemination.
ORCID	0000-0001-8643-5061
% FTE Commitment	10.00%

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Joint Lead Applicant

Full Name	Dr Helen Cramer
Organisation	University of Bristol
Department	Centre for Academic Primary Care
Proposed Role	Chief Co-lead investigator; overseeing adaptation of PATH; referral pathway lead; qualitative lead including theory of change; overseeing recruitment.
ORCID	0000-0001-5989-2638
% FTE Commitment	10.00%

Co-Applicants

Full Name	Dr Victoria Vickerstaff
Organisation	Queen Mary University of London
Department	Wolfson Institute of Population Health
Proposed Role	Senior statistician and trialist input; overseeing statistical analyses and health economics data.
ORCID	0000-0002-3119-670X
% FTE Commitment	7.50%

Full Name	Dr Roxane Agnew-Davies
Organisation	Domestic Violence Training Ltd
Department	University of Bristol Centre for Academic Primary Care
Proposed Role	Overseeing the training and supervision of DA practitioners; advising on changes to PATH and training resources.
ORCID	0000-0002-2990-755X
% FTE Commitment	10.00%

Full Name	Dr Alison Gregory
Organisation	University of Bristol
Department	Centre for Academic Primary Care
Proposed Role	Supervision of DA practitioners; therapeutic skills; advising on theory of change development of A-PATH and qualitative perspectives.
ORCID	0000-0002-4768-1574
% FTE Commitment	10.00%

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Full Name	Mrs Amanda Wynn
Organisation	University College London
Department	Division of Psychiatry
Proposed Role	Safeguarding lead; overseeing safeguarding issues for participants monthly; advising on training of DA practitioners; expertise in sexual abuse of older women.
ORCID	0000-0002-9788-3326
% FTE Commitment	10.00%

Full Name	Professor Sarah Wydall
Organisation	Swansea University
Department	Criminology Sociology and Social Policy
Proposed Role	Overseeing development of A-PATH; engaging with policy makers; national expert on DA crisis and recovery models for older people.
ORCID	0009-0004-9611-6530
% FTE Commitment	7.00%

Full Name	Dr Claire Hawcroft
Organisation	University of Bristol
Department	Centre for Academic Primary Care
Proposed Role	Clinical Lead; primary care pathways for older women affected by domestic abuse.
ORCID	0000-0002-9295-5044
% FTE Commitment	5.00%

Full Name	Ms Marion Goodchild
Organisation	PPIE based in England
Department	UCL
Proposed Role	Lead of Patient and Public Engagement strategy; overseeing changes to PATH and training materials.
ORCID	
% FTE Commitment	10.00%

APPLICATION DETAILS

Scientific Abstract

Domestic abuse (DA) affects many women over the age of 60 and has a profound effect on their well-being. Older women face distinct and often overlooked barriers to seeking help, such as ageist attitudes, stigma and assumptions that DA does not happen in later life. Older women are more likely to be financially dependent or live with the perpetrator, making it harder to access support and cope with the abuse. Psychological Advocacy Towards Healing (PATH) is a trauma-informed intervention delivered by DA practitioners that improves psychological outcomes in younger women. However, its effectiveness for older women remains unclear.

In this programme of research, our aim is to adapt the PATH programme (A-PATH) to meet the unique needs of older women, and assess its feasibility and acceptability for a future large scale randomised controlled trial (RCT).

We will address two key research questions:

a) is it possible to adapt PATH in a way that is accessible to a diverse range of older women affected by DA to empower and support their wellbeing, and recovery?

b) is it feasible and acceptable to deliver A-PATH in primary care, and community settings?

Our mixed-methods programme of research will include three interlinked Work Streams, with a Lived Experience Advisory Panel (LEAP) contributing throughout.

Work Stream 1: We will conduct two survivor-led focus groups (n = 16) with older women affected by DA to tailor the components of PATH to better meet the needs of older women. Practitioner focus groups (n = 10) will further inform the adaptation of PATH, help identify training needs, and enhance practitioners' ability to recognise the needs of older women.

Work Stream 2: Using individual interviews with survivors (n=10), and shadowing helpline phone calls (n=10) through our partner Hourglass, we will explore the barriers older women face in accessing support. We will work with frontline professionals and clinicians (n = 15) to identify and develop referral pathways, that improve visibility and accessibility of services for older women in both primary care and community settings.

Work Stream 3: We will test the feasibility and acceptability of A-PATH in a pilot RCT (n = 76 women), by randomly allocating 38 women to DA advocacy and 38 to DA advocacy plus A-PATH. We will measure rates of recruitment, randomisation, retention, and participant engagement with the intervention. We will identify and administer measures which we think may be suitable as primary and secondary outcomes in a full trial at baseline, 4, 8, and 12 months. This will include: psychological distress, depression, and post-traumatic stress. We will assess the acceptability of collecting health-related quality of life data to inform a future economic evaluation and conduct interviews with older women and practitioners to explore lived experiences and contextual factors influencing outcomes.

We will work closely with community providers to engage women affected by multiple inequalities and explore how these intersecting factors influence access to support services. Findings will be shared through a series of accessible reports tailored for diverse audiences, alongside a public engagement event to communicate key learning. If feasible, this study will provide the foundation for a future large scale RCT evaluating the effectiveness of the adapted PATH, generating much needed evidence to improve trauma-informed care and support for older women affected by DA.

Plain English Summary

Background to the research

Some women over the age of 60 are living with domestic abuse, which means they are being hurt or mistreated by someone close to them, such as a partner or ex-partner. This abuse can include physical harm, made to feel scared or upset, being controlled, or stopped from making their own choices. Sometimes, this abuse lasts for many years and may involve emotional, sexual, or financial harm. Most domestic abuse support services are aimed at younger women and may not fully understand or meet the needs of older women especially those who are still living with the person causing them harm. This

can make it much harder for older women to get the help they need to stay safe.
Aim of the research
Our research aims to help provide better support for older women who are living with abuse. Women who have lived through domestic abuse have told us that they want a safe and supportive space to share memories of what happened and begin planning for their future. In this project, we will work with a small group of older women to find out if a programme called 'Psychological Advocacy Towards Healing' (PATH) can help them stay safe and feel better. PATH has already been shown to help younger women (under 60) who have experienced abuse.
Research plan
We will work with older women and their supporters to make the PATH programme better suited to their needs. We will learn about the challenges they face when seeking help and work closely with health and support workers to find the easiest ways for them to get the support they need. We will invite a group of older women to take part in a study to try out the PATH programme. The group will be split into two smaller groups at random. This means no one will choose which group they are in; it will be decided by chance. One group will be offered the usual support offered to survivors of domestic abuse, such as meetings with a trained support worker. The other group will get this same support, plus weekly one-to-one therapy sessions for 8 to 12 weeks. This will help us understand whether PATH can benefit older women and what changes may be needed to make it more suitable. At the end of the study, we will ask women in both groups to share their thoughts about what worked well and what could be better, to help plan the next steps.
Working with diverse people and communities
Older women of different ages and backgrounds who have lived experience of abuse, have played a key role in shaping the project, and will be involved throughout to ensure the programme meets everyone's needs. An older woman researcher with personal experience of domestic abuse is working closely with us to make sure the study truly reflects what support older women need. The national charity Hourglass, which works to end the abuse of older people, will help our study to ensure it is shaped by the needs and voices of older women.
Knowledge mobilisation
When the study is finished, our public contributors will guide us in sharing what we have learned with the women we worked with, as well as with doctors, nurses and organisations that support women affected by abuse, including those who work with older women. If this small study testing the A-PATH programme shows it could be helpful for older women, we will apply for further funding to carry out a larger study with more people. Later, we hope to work with policymakers to help bring lasting improvements in the well-being and safety of older women harmed by domestic abuse.

Changes from Previous Stage
Thank you for these very helpful comments. Below is our detailed plan of all changes for Stage 2 based on your feedback:

1. The applicants should describe how retention will be investigated in the feasibility study.
Thank you for this helpful comment which has been addressed. Stop/go criteria are now specified (see section primary outcomes). Retention will be a key focus of our feasibility randomised controlled trial. As described in our full proposal, we will collect both quantitative and qualitative data to assess retention. This mixed-methods approach will enable us to assess the feasibility of achieving adequate retention (see pages 7-9).

2. The applicants should describe how they will be encouraging recruitment from non-English speaking and ethnic communities.
Thank you for this very important comment. As described in page 18 to encourage recruitment from non-English speaking and ethnically diverse communities, we will implement inclusive and culturally sensitive engagement strategies throughout the study. Recruitment materials (i.e. participant information sheets, posters) will be reviewed for cultural appropriateness. We will work in partnership with frontline services that have established trust within ethnic minority communities to raise awareness of the study. Involving community representatives in the co-design of recruitment strategies ensures they are responsive to

cultural needs. These measures will foster equitable access and participation from under-represented groups, ultimately improving the relevance of the adapted PATH intervention.

3. The applicants should correct the timeline for workstream 3.
Thank you for pointing out this error. This has now been corrected.

4. The committee requires reassurance that the study can be delivered within the proposed time and budget.
Thank you for your valuable feedback. We are confident in delivering the study within the proposed timeframe and budget, supported by our proven track record. Our team has extensive experience managing complex intervention studies in areas such as dementia, end-of-life care, and domestic abuse, having led multiple trials with similar deadlines. We have strong partnerships with key organisations (e.g., Hourglass, Women's Aid, Age UK, SafeLives) that will actively support recruitment. We will work closely with NHS Trusts that have demonstrated leadership in domestic abuse research and practice (e.g., Camden and Islington NHS Trust – Pathfinder Project; Central and North West London NHS Foundation Trust – routine screening implementation) to strengthen recruitment through collaboration and shared learning. The project includes continuous monitoring and oversight to keep the study on track and within budget, maximising quality and impact. Following recommendations for involving underserved groups, we have expanded to ten sites and a recruitment period of eight months—an approach shown to improve diversity and make study findings more relevant and impactful.

5. The applicants should describe the intervention development.
Thank you for this comment. We have now described the development of A-PATH in detail in section Work Stream 1. The development of the intervention will follow established best practice for complex intervention development (drawing from the Medical Research Council framework). It will involve a phased, collaborative approach with experts by experience, DA practitioners, clinicians and third sector to ensure it is relevant, acceptable, and tailored to the needs of older women affected by DA.

6. The plain English summary should be rewritten for a general audience.
Thank you for this helpful feedback. We have revised the Plain English Summary to ensure it is clear, and accessible. The revised version now explains in lay terms the purpose of the study, what the research will involve, and how it could benefit older women affected by DA. We have involved members of our Public and Patient Involvement (PPI) group, in reviewing the revised summary to ensure it is understandable to a general audience. Our plain language summary has been developed with input from our DA practitioner co-applicant, NIHR research design lay members, and experts by experience consulted through our partner organisation, Hourglass.

7. The applicants should clarify the rationale for the 4-month follow-up being the main outcome measurement point.
Thank you for this very important feedback. We consulted with lived experience experts to inform our choice of primary outcome timepoint. They strongly emphasised the need to allow sufficient time for meaningful and sustainable change to emerge given the complex, long-standing nature of DA in later life. Their insights highlighted ensuring participants have adequate time to complete A-PATH and begin applying its content to their daily lives. This longer timeframe aligns with other DA intervention RCTs, and NIHR feasibility study guidance, which support the importance of assessing medium-term impacts. For the future full-scale trial, we will extend follow-up to 12 months as our primary outcome timepoint, consistent with the approach used in our REPROVIDE trial, to enable measurement of longer-term recovery outcomes.

8. The applicants should clearly describe the definition of adherence that they intend to use in the study.
We now describe the definition of adherence that we will use in our feasibility study. This is based on our previous PATH trial, prior research in the area, our lived experience study, and feedback by our PPI group. Details of our approach to adherence is described in detail in page 9.

9. The applicants should give more detail about sample size calculations.
We now expand on our sample size justification (see Work Stream 3, pages 8-9). This is based on guidance for feasibility trials where the focus is on estimating parameters such as recruitment, retention, and adherence with sufficient precision (NIHR RfPB Guidance, 2023).

Working with People and Communities Summary
We define our community as women aged 60 and over who have experienced domestic abuse, along with their families, supporters, and the organisations that contribute to their recovery and well-being. Our community-engaged research approach is grounded in the principles of equity, inclusivity, shared learning, and collaborative decision-making, bringing together lived experience experts, practitioners, third sector partners, clinicians and researchers.
We will work in close partnership with older women and the professionals who support them to co-produce knowledge and promote meaningful involvement in all stages of the research. Our approach will ensure that older women's voices are central to shaping the intervention, and improving referral pathways. We will evaluate the strength of our community engagement, by monitoring and measuring:
(1) the number and diversity of community contributors (e.g., local services, third sector organisations, gatekeepers);
(2) engagement across the study, noting retention and consistency of community voices in shaping study procedures, materials, and dissemination plans;
(3) the perceived impact of involvement, assessed through follow-up interviews with older women and practitioners.
From the earliest stages of our study design, we engaged a core group of older women with lived experience, who form our Lived Experience Advisory Panel (LEAP). This panel will collaborate with the research team to co-design focus group agendas, participate in facilitation, and play a central role in interpreting study data. The LEAP will be chaired by our co-applicant with lived experience, who will help shape the group's focus and priorities over the course of the study. The LEAP group will convene at key milestones of the project to contribute to:
a) adapting PATH content;
b) identifying referral pathways and potential barriers to access;
c) refining the core principles underpinning the intervention;
d) help to plan and guide the feasibility study.
To support participation, we will use safe, accessible, and trusted local venues such as Age UK centres, women's groups, and GP practices to host focus groups, LEAP meetings, and PATH sessions. Informed by Goodman et al.'s 2017 community engagement toolkit, we will ensure that all materials are accessible, culturally appropriate and relevant to the preferences and needs of older women affected by domestic abuse.
We are committed to ensuring mutual benefit in all aspects of community involvement. Contributions will be recognised through participation vouchers, flexible scheduling, and practical support to reduce barriers such as digital exclusion. We will also offer lived experience experts opportunities for training in research to support their ongoing development and meaningful involvement in the study (through training in the PATH model and qualitative research methods).
Community members will receive letters of recognition to support future engagement in research. Our LEAP, led by our peer researcher, will take a lead role in co-producing plain language summaries of our findings for community dissemination. At the end of the project, we will host a public engagement event with our partners (Hourglass, Age UK, Women's Aid) to share results, raise awareness, and foster wider social change. Rooted in co-production, this study will prioritise safe, inclusive participation to ensure the research remains relevant, and respectful to the communities it serves.

FUNDING COMMITTEE FEEDBACK

:

NIHR Health Technology Assessment Programme

Feedback

1. The applicants should be explicit about whether a theory of change model is going to be used given that this was suggested in the research specification.
2. The current timeline is overly ambitious and should be reconsidered.
3. The project would benefit from the involvement of a clinical trials unit that can provide relevant delivery expertise.
4. The committee notes that the above points will lead to an increase in costs, however this should be limited to a modest increase.
5. Further consideration of how the inclusion of a diverse range of participants can be ensured so that different groups are represented.
6. The committee suggests that a suitable minimum number of participants is set for the focus group work and interviews to ensure that they produce a meaningful output.
7. Data on retention at 12 months should be collected.
8. The applicants should incorporate the changes proposed in response to the external reviewers' comments.

Response to feedback (completed by the research team)

Response to feedback
Thank you for this helpful feedback to our application. Please see below our detailed response.
1. The applicants should be explicit about whether a theory of change model is going to be used given that this was suggested in the research specification.

Thank you for this important point. In adapting APATH (Adapting Psychological Advocacy Towards Healing), we will draw on the Psychosocial Readiness Model (PRM) of change (Cluss et al., 2006). This model is well suited to older women affected by lifelong abuse, as it recognises how psychological, situational, and social factors influence readiness for change, and provides a framework to tailor our intervention so that it promotes empowerment, engagement, and sustained recovery. We will refer to the theory of change at the start of the study as we adapt our intervention and set the research up. We will return to the theory of change when we have collected our data. If the pilot has shown acceptability and feasibility for a full trial, we will revise the theory of change in preparation for our full trial application.

2. The current timeline is overly ambitious and should be reconsidered.
We have reviewed and adjusted the project timeline, extending it by six months to ensure feasibility (full details appear on our Revised Time plan for the study). This revised timeframe will allow us to collect 12-month follow-up data for all participants and support the thorough completion of all project activities.
3. The project would benefit from the involvement of a clinical trials unit that can provide relevant delivery expertise.
Thank you for this helpful suggestion. We agree that clinical trials expertise will strengthen the project. The lead applicant will work closely with UCL's Priment Clinical Trials Unit (CTU), which will provide guidance on study design, data management, and delivery processes. She will join the Priment CTU Steering Group (https://www.ucl.ac.uk/priment/priment-steering-group), present progress at regular

meetings, and receive ongoing support from the Head of Clinical Trials Operations to ensure methodological rigour and effective delivery. The lead applicant will also attend regular departmental meetings for Chief Investigators of clinical trials.
In addition, our team includes four academics (VO, HC, VV, RAD) with prior experience in the delivery and management of NIHR-funded clinical trials in areas such as dementia, domestic abuse, and end-of-life care.
4. The committee notes that the above points will lead to an increase in costs, however this should be limited to a modest increase.
Thank you for this observation. We recognise that the revisions, including the extended timeframe result in a cost increase; however, we have been very careful to limit this to a modest increase, ensuring that all additional costs are fully justified and essential to delivering a high-quality study.

5. Further consideration of how the inclusion of a diverse range of participants can be ensured so that different groups are represented.

Thank you for this important point. We are committed to ensuring the inclusion of a diverse range of participants to reflect the varied experiences of older women affected by domestic abuse. Recruitment will be supported through community and voluntary sector organisations, including Hourglass, Women's Aid, and groups representing ethnically and culturally diverse communities. As part of this project, we also aim to develop a community network specialising in supporting diverse groups—such as Shakti Women's Aid, The Angelou Centre, and the London Black Women's Project—to enhance inclusivity and ensure the intervention is relevant and accessible to all participants. We will also work closely with Aanchal, who support South Asian women affected by domestic abuse.
In addition, we will draw on the local resources and expertise available at each study site to strengthen engagement and inclusion. For example, in Bristol, we will collaborate with local health inequality community champions, including the BNSSG ICB Health and Social Care Research Ambassadors and the new Deep End Health Inequality Fellows. These roles are specifically designed to support research studies in increasing diversity and inclusion, and their involvement will help ensure that recruitment strategies are locally grounded and responsive to the needs of underrepresented groups.

6. The committee suggests that a suitable minimum number of participants is set for the focus group work and interviews to ensure that they produce a meaningful output.
Thank you for this helpful suggestion. We have now specified minimum numbers to ensure meaningful outputs:
Work Stream 1 – Adaptation of PATH
• Each focus group with survivors will include 6–8 participants to ensure meaningful input into exploring the intervention content and adapting it to the needs of older women.
• Each focus group with domestic abuse practitioners will include 6–8 participants to assess and adapt the original training package.
Work Stream 2 – Developing referral pathways
• We will conduct a minimum of 10 interviews with women with lived experience to understand barriers

and facilitators to accessing services.
• Each stakeholder focus group will include 6–8 participants to explore the suitability of current referral pathways.

Work Stream 3 – Follow-up interviews of A-PATH
• We will conduct a minimum of 15 interviews with women with lived experience to understand their experiences of A-PATH and identify areas for improvement. We will interview a minimum of 10 women that receive usual care.
• We will conduct 6–8 interviews with domestic abuse practitioners delivering the intervention.
This approach will provide sufficient diversity and depth to capture a broad range of perspectives. We will also monitor data saturation throughout the data collection process to ensure that sampling continues until no new themes or insights emerge, thereby strengthening the robustness and credibility of the findings.

7. Data on retention at 12 months should be collected.
Thank you for this important point. We will collect retention data at 12 months for all participants to monitor engagement, assess the feasibility of sustained participation, and inform the long-term evaluation of the intervention. Collecting 12-month data is particularly valuable for this group, as older women affected by domestic abuse may experience complex and evolving needs over time. Tracking retention at this stage will help us understand factors that influence continued engagement, identify barriers to sustained support, and provide insights into the longer-term relevance and acceptability of the intervention within this population.
8. The applicants should incorporate the changes proposed in response to the external reviewers' comments.
All of the proposed changes will be accommodated. We have also written to the Women's Institute and hope to arrange a meeting with them very soon including local representatives. In addition, we will engage with relevant contacts in Northern Ireland, Scotland, and the East Midlands to ensure broad input and representation in the project.

References
Cluss PA, Chang JC, Hawker L, Scholle SH, Dado D, Buranosky R, Goldstrohm S. The process of change for victims of intimate partner violence: Support for a psychosocial readiness model. Women Health Iss. 2006;16(5):262-74.

Other changes to the application

No changes have been made to the application because of other factors.

INTELLECTUAL PROPERTY (IP) FEEDBACK

:

Is Background IP owned by someone other than the Contractor (a Third Party) used in the delivery of the research?

No

Are any items of Foreground IP intended to rest somewhere other than the Contractor?

No

Other changes to the application

No other changes have been made to the application because of other factors.

SUMMARY BUDGET

Justification of Costs
Staff Costs

1. Chief Investigators and Co-Applicant Team
Dr Vasiliki Orgeta (Lead for WS1 and WS3) and Dr Helen Cramer (Lead for WS2) will have overall responsibility for the project's delivery and management. Dr Orgeta will lead recruitment at the London sites and oversee recruitment across all sites (costed at 10% FTE). She will manage the NHS Ethics and HRA approvals, supported by colleagues at Noclor and the UCLH/UCL Joint Research Office. She will lead all site initiation meetings with all recruitment centres during the set-up phase (see Timeplan; Months 0–9) to ensure timely and coordinated study delivery. She will also engage community-based hubs to support study recruitment and outreach. This hands-on approach will support smooth operational start-up and foster strong collaborative relationships critical to recruitment success for each site. She is costed at 10% for the first 30 months, and at 5% for the last 6 months of the study. Dr Cramer will lead recruitment at the Bristol site and will lead WS2, focused on developing referral pathways (costed at 10% FTE for 30 months and 5% for the remaining 6 months).

Professor Sarah Wydall (Swansea site lead, WS3 Co-Lead, co-applicant) will oversee recruitment at the Swansea site and co-lead the WS3 qualitative process evaluation of lived experiences for survivors and practitioners (costed at 7% FTE for 24 months). Dr Roxane Agnew-Davies (co-applicant, Co-Lead of WS1) will lead on A-PATH training and supervision, contributing to the adaptation of training and A-PATH materials (costed for 57 days). Dr Alison Gregory (co-applicant, WS2 Co-Lead) will support the development of community referral pathways (WS2) and lead on third sector engagement partnerships with Women's Aid and SafeLives (costed for 40 days).

Dr Victoria Vickerstaff (statistical and health economics lead; co-applicant) will oversee statistical and economic evaluation (costed at 7.5% FTE for 24 months), supported by a junior researcher (employed by UCL). Marion Goodchild (lived experience co-applicant) will lead the PPI strategy and chair the Lived Experience Advisory Panel (LEAP). She will guide the adaptation of PATH, advise on recruitment, and review all public-facing materials (costed for 20 days).

2. Research Staff Employed by UCL
We will employ a senior researcher (50% FTE for 22 months and 25% for 10 months; Grade 7) with a background in social work and expertise in domestic abuse and safeguarding older adults. This role will also help build early-career research capacity, with the post-holder undertaking a structured training plan and receiving mentorship by senior leaders in the field. Managed by Dr Orgeta and Dr Cramer, they will support recruitment and data collection across all Work Streams and assist WS Leads with project coordination. From our experience data collection will primarily be conducted over the phone to prioritise accessibility, flexibility, and participant comfort. Where feasible and appropriate, we will also offer in-person data collection through local Clinical Research Network (CRN) sites to support participants who prefer face-to-face engagement or need additional assistance. This blended approach aims to promote inclusivity and optimise recruitment and retention.

Dr Phuong Leung (Senior Research Fellow, UCL), who led the previous lived experience study, will co-lead the process evaluation interviews of WP3 (10% FTE over 16 months) alongside Professor Wydall and support final dissemination of academic outputs.

Total Staff Costs: £168,895.96

Dr Claire Hawcroft (Honorary Researcher, University of Bristol) will lead on the distress protocol and

advise on primary care referral pathways. Amanda Warburton (Honorary Researcher, UCL Psychiatry) will oversee recruitment of DA practitioners, develop the safeguarding protocol, and lead on safeguarding procedures throughout the trial.

Non-Staff Costs
1. Data Capture, Randomisation, Training, and Participant Vouchers
We have allocated costs for randomisation via Sealed Envelope (100 randomisations). REDCap will be used for data capture (annual fee waived for UCL). We have included training costs for our key outcome measures, and the fee for registering our trial (ISRCTN). All participants will receive £25 vouchers as recognition for their contribution.

Total: £5,886.40

2. Dissemination
We have included costs for a podcast to be co-produced with UCL Media, a final dissemination event, and attendance at a national conference.

Total: £7,360.00

3. SoECAT Site-Level Research Costs
As outlined in the SoECAT, these include research activities essential to study delivery, including recruitment, consent, and data collection.

Total: £10,218.08

4. Estate, Infrastructure Technician Costs, and Other Indirect costs
These amount to £143,023.48 comprising estate costs, infrastructure technician costs, and indirect costs.

Total Funds Requested: £335,383.92

NIHR Health Technology Assessment Programme

ACKNOWLEDGEMENT AND CONFLICTS

I have read and understood the terms of applying as Lead Applicant and accept this role	✓

Potential Conflicts	There are no known conflicts of interest.

A-PATH: Adapting Psychological Advocacy Towards Healing for older women affected by domestic abuse

Background and rationale

Data published by Age UK in 2023 indicate that more than 375,000 older women aged 60 to 74 experience domestic abuse (DA) in England and Wales (1). The 2025 Femicide Census has revealed that one in eight women killed by men in the past 15 years was aged over 70 (2). DA can lead to serious physical health injuries for older women (3), often associated with prolonged recovery periods and an increased risk of mortality (4, 5). DA has profound emotional and psychological impacts, including increased severity of anxiety and depression (4), which significantly diminish older women's well-being and quality of life (6). Despite a growing number of older women experiencing DA, reports published by our partner Hourglass (7, 8) highlight persistent gaps in support and mental health provision for older women (9).

DA can expose older women to a range of harmful behaviours that are frequently underreported and poorly recognised within services (10, 11). These may include physical and emotional abuse – such as sustained verbal belittlement and psychological manipulation – as well as sexual abuse involving coercive or non-consensual acts, often occurring within long-term relationships (12, 13). Financial abuse is also common, typically involving the misuse of pensions, or the restriction of access to personal funds (14). Controlling behaviours – such as limiting access to healthcare or isolating individuals from social networks – further undermine older women's autonomy and well-being (14). These forms of abuse are often exacerbated by generational norms, such as the expectation that older women should endure hardship, and by systemic barriers, such as digital exclusion and insufficient training among professionals, which together limit access to timely support and care (15, 16).

The unique barriers faced by older women affected by DA are frequently overlooked in mainstream service provision and academic research (17). Abuse may span decades, exposing women to cumulative trauma, and become embedded in long-term relationships, making it harder to recognise or disclose (18). Many women may not recognise controlling behaviour or financial coercion by a long-term partner as abuse, particularly when such patterns have been normalised over time (19). For some older women, disclosing abuse may carry a significant risk of losing their home, financial security, or contact with grandchildren (20). Living with a chronic condition or frailty, increase dependence on the perpetrator, thereby reducing opportunities for older women to seek help (4).

Health and social care professionals sometimes miss signs of abuse, often attributing injuries or distress to age-related conditions or frailty rather than recognising them as intentional harm (9, 21). Our qualitative study has shown that digital exclusion—caused by limited access to technology or digital literacy (22)—prevents many older women from accessing support, or engaging safely with remote interventions (18). These factors not only increase older women's vulnerability to sustained and repeated abuse, but also highlight the need for targeted support (23), so that older women can get the help they need (18).

Psychological interventions are a vital component of the care pathway for women experiencing DA (24, 25), addressing the significant mental health impacts such as

trauma, and depression (19, 26). While these interventions are typically accessed once immediate safety needs have been met (15, 27), there is growing evidence supporting their value earlier in the help-seeking journey, including for women who remain in abusive relationships but are beginning to engage with services (15). In these contexts, psychological support can help build emotional resilience, strengthen coping strategies, and support informed decision-making around safety and self-care (25).

Psychological interventions that are not tailored to meet the specific needs of older women are less likely to be useful to female survivors and may even be harmful (28). Such harm can include retraumatisation, feelings of alienation or invalidation, and reduced engagement with services perpetuating health inequalities faced by older women (20, 27). Our review reporting on the clinical effectiveness of psychological treatments for older people exposed to trauma, found no interventions for older women who have experienced DA (29). This project aims to address this critical service gap by adapting an existing evidence-based psychological intervention - Psychological Advocacy Towards Healing (PATH) - originally developed for women of childbearing age surviving abuse (30, 31), to meet the unique needs of older women.

Developed by Agnew-Davies and colleagues (32), PATH integrates cognitive behavioural therapy (CBT), experiential and feminist theoretical frameworks to equip women with practical skills for managing the psychological distress associated with abuse (31). Although the PATH intervention has demonstrated effectiveness in reducing psychological distress and depressive symptoms in women aged 18 to 67 (30), its suitability for older women remains unclear, particularly in addressing their distinct experiences and the specific barriers they face when seeking help (20). Our research has highlighted a range of unique and intersecting factors that make it harder for older women to access support and cope with abuse (9, 11). These include digital exclusion, frailty and social isolation (33), which are often exacerbated by ageism within health and social care services, and intensified by prolonged coercive control that restricts social networks (4). As part of this study, we will also seek to examine the need for and, as necessary, develop specific referral pathways for older women who experience DA. Improving access is an essential pre-requisite to more older women getting psychological support and more equitable access and service provision (25).

Although PATH promotes skills to enhance women's self-esteem, assertiveness, and resilience – core components of long-term recovery and empowerment (31), it lacks adaptation for age-related factors such as living with frailty and the complexities of lifelong abuse or late-onset abuse (13, 19). It does not address for example, barriers commonly faced by older women, such as digital exclusion, generational stigma around help-seeking, and dependence on abusers for care or housing (18, 19, 22). As a result, the standard PATH intervention may not be accessible, acceptable, or effective for this population without modification (34, 35). Adapting PATH to reflect the specific needs and circumstances of older women could significantly enhance its reach and relevance, extending the benefits of this evidence-based intervention to a significantly underserved group (31).

In focus group discussions with older women survivors, participants emphasised the value of accessing psychological therapies that can validate their experiences, and reported that they sometimes receive practical and emotional support through local statutory DA practitioners. Currently however, DA practitioners are not trained to deliver specialist psychological support, limiting the scope of care available to older women. In this project our aim is to train DA practitioners to deliver the adapted PATH intervention (A-PATH), enabling them to provide psychologically informed support alongside advocacy as usual (26).

Aims and objectives

Our overarching aim is to improve the mental health of older female survivors of DA, and support their well-being and recovery. To achieve this, we will: i) adapt the core components of the existing PATH intervention to ensure they are tailored to the needs and preferences of older women, (ii) strengthen and tailor referral pathways to enhance accessibility with support services, and, iii) determine the feasibility and acceptability of the adapted PATH intervention and trial design.

The objectives are to:

1. Adapt the PATH intervention specifically for older female survivors, including the development of appropriate referral pathways;
2. Identify the main variables that may influence the design of a full-scale trial, including factors affecting the evaluation of the cost effectiveness of this intervention;
3. Determine the acceptability and feasibility of the adapted PATH and trial design from the perspectives of clients and advocates and;
4. Explore the experiences and perceptions of those receiving and delivering the adapted intervention and its context.

Why is this research important in terms of improving the health and wellbeing of the public and health care services?

Older women affected by DA often access support through NHS services such as general practice (GP), emergency departments and hospital wards, where abuse may be disclosed or suspected during consultations related to physical or mental health concerns (36). The impact of DA in later life is often far-reaching, long-lasting and cumulative—exacerbating existing health conditions and contributing to the onset or worsening of mental health disorders (3, 6). Prolonged exposure to abuse can increase vulnerability to chronic health conditions, such as cardiovascular disease, musculoskeletal pain (37, 38) and elevates risk of frailty (39). Timely and tailored psychological interventions delivered by DA advocates have the potential not only to alleviate immediate psychological distress but also to reduce long-term health risks and subsequent service use among older women (40).

What is the evidence that the intervention is ready for HTA evaluation?

PATH is a manualised, trauma-informed intervention grounded in cognitive-behavioural principles, which is effective in reducing psychological distress and depression for women experiencing DA (25). This existing evidence base highlights the intervention's readiness for further adaptation and targeted evaluation, particularly for underserved groups such as older women (35). Preliminary work by

our team that included a systematic review of psychological interventions for older people exposed to trauma has identified a lack of tailored psychological interventions for older women affected by DA (29). Additionally, our Patient and Public Involvement (PPI) activities have consistently highlighted a significant gap in age-appropriate services and a pressing need for psychological support that recognises the unique experiences and barriers faced by older women.

Research questions

Main research question: Is it possible to adapt PATH in a way that is accessible to a diverse range of older women to empower and support their wellbeing and recovery?
1a) What does an adapted PATH intervention look like, and what types of formats and delivery are wanted?
1b) What are key stakeholders' views about current referral pathways?
1c) What are the barriers and facilitators for older women needing help?
1d) What solutions can be applied to optimise referral pathways for older women?
1e) Is it feasible to deliver A-PATH in primary care and community settings?
1f) What is the feasibility of collecting outcome measures and study procedures?
1g) What is the acceptability of A-PATH and trial conditions to older women and DA practitioners?
1h) How can the PATH intervention be adapted to effectively support the diverse needs of older women affected by DA including those from ethnic minority backgrounds and varied geographic settings?
1i) How can a revised fidelity framework ensure consistent and culturally responsive delivery across different contexts?

Methodology/Plan

The programme will be delivered over 30 months and will comprise 3 interlinked work streams (WSs). We will adapt PATH with the support and input of a Lived Experience Advisory Panel (LEAP). PPI will be embedded throughout all stages of the research, including through the LEAP panel. To ensure diversity and inclusivity, we will proactively recruit LEAP and PPI members through outreach specialist services that support older women from a range of ethnic, cultural and sexual identity backgrounds. One of our co-applicants brings lived experience of DA and will play a key role in project management, leading on PPI recruitment and co-facilitating the LEAP panel.

Design

We will use a mixed-methods design, a pilot randomised controlled trial incorporating acceptability and feasibility, and a process evaluation with embedded qualitative work (41) to:

a) WS1 - adapt the components and materials of PATH including practitioner training resources, and handouts for service users
b) WS2 - identify suitable and accessible referral pathways to facilitate older women's access to psychological support and advocacy services, and
c) WS3 - pilot test the feasibility and acceptability of A-PATH using a RCT design, alongside a nested qualitative sub-study to explore the acceptability,

experiences, and contextual factors influencing engagement with the intervention.

Work Stream 1

Part 1. *Adaptation and development of A-PATH*

In two focus group meetings (each with up to 8 survivors purposefully recruited to reflect diversity in age and ethnicity), we will explore the content of the intervention, elicit preferences for key topics (e.g., *Making sense of it all, Flashbacks and intrusive memories, Injured self-esteem*), and identify any gaps relevant to the needs of older women. The focus groups will be used to explore the current content and delivery of the PATH intervention, with the aim of understanding why it may not fully address the specific needs of older women (42). Discussions will identify specific elements of the intervention in relation to older women's experiences, as well as their preferences and barriers to accessing support. Prior qualitative findings and relevant literature (29) will be used to inform and enrich these discussions, ensuring they are grounded in existing evidence and lived experience.

While the PATH intervention has primarily been delivered to women who have left abusive relationships (30), we recognise that many older women remain in an abusive relationship due to complex factors such as dependency, care needs or housing barriers (18, 19). As part of this study, we will explore whether PATH is usable and acceptable for older women who are still in abusive relationships, and identify what adaptations may be necessary to ensure it is safe, appropriate, and responsive to their specific needs and circumstances. We will actively involve older women with lived experience in informing and shaping the adaptation of PATH in a meaningful and respectful way (42). When developing the intervention materials, we will draw on the linguistic and cultural expertise of our community partners to ensure accessibility and relevance for women whose first language is not English (43).

Our focus groups will be supported by our LEAP experts (44), and will invite older women survivors of DA to suggest improvements to PATH. The revised intervention will then be presented in a roundtable discussion with key stakeholders (representatives from Hourglass, Women's Aid, Age UK, DA practitioners; n = 8) to gather further feedback (35). This final stage will inform refinements to the content, materials, and logic model (see page 19). All components of the A-PATH intervention will be summarised using the Template for Intervention Development and Replication (TIDieR) checklist to support transparency and future scalability (45).

Preparation for the feasibility RCT

We will field-test our intervention with 4 survivors, to explore the practicalities of different delivery modes - telephone, online, and in-person - allowing us to identify any unaddressed barriers. In partnership with Solace Women's Aid (22), we will explore strategies to mitigate digital exclusion, which remains a significant barrier for older women. This could include for example offering non-digital options (e.g. phone, face-to-face, printed materials), providing basic digital skills support where appropriate, and working with community partners to ensure access to trusted spaces with digital resources. These strategies will be co-informed by the lived experience of older women to ensure accessibility, safety, and choice. We will also

explore practical factors influencing session attendance, such as mobility limitations, transport barriers and caring responsibilities (46).

Part 2. *Adaptation of training package for DA practitioners*

We will conduct two focus groups with DA practitioners (n=10), including those with and without experience working with older women, to assess the original PATH training package (30). To ensure the adapted PATH intervention is responsive to the needs of survivors from diverse ethnic, cultural, socioeconomic, and sexual identity backgrounds, we will also examine the inclusivity of the practitioner workforce, considering how diversity among practitioners can support more equitable and effective delivery. This includes exploring who delivers the intervention, how they are trained, and whether a more inclusive and representative workforce could enhance engagement with A-PATH. We will consider how practitioner background, cultural competence, and life experience influence delivery, and identify opportunities to strengthen training to support a more diverse, trauma-informed and inclusive approach.

Based on this input, we will develop and deliver a flexible training programme using a combination of online and face to face sessions, incorporating interactive and experiential learning methods. Each practitioner will receive a comprehensive training manual (47) that outlines session formats – including focus setting, targeted topic discussions, and progress evaluation (48) – alongside concrete examples to support delivery. These examples will guide practice in addressing common presenting problems identified among older women (49). To support skills development, we will provide one-to-one booster sessions, reflecting practitioner feedback and evidence that such sessions enhance the effectiveness of DA training (15, 49) by strengthening clinical skills (50). Group supervision will be provided throughout the delivery of A-PATH, using a hybrid model to accommodate online and in-person participation. Data from both sets of focus groups (Part 1 and Part 2) will be analysed using framework analysis (51) to identify key adaptations needed to ensure the intervention is appropriately tailored to the specific needs of older women.

Work Stream 2

Identifying suitable referral pathways

This Work Stream aims to improve access to psychological support for older women affected by DA by developing clear, setting-specific referral pathways across general practice, emergency departments, acute hospital wards, and community-based services (52). These settings are often the first point of contact for older women, where disclosures may occur in the context of experienced physical or mental health concerns. However, responses can be inconsistent, and older women's specific needs are frequently overlooked. Creating clear, setting-appropriate referral pathways is critical to ensuring timely, safe and supportive access to psychological interventions like A-PATH. This approach can help address existing service gaps and reduce missed opportunities for intervention. Informed by early stakeholder engagement, this will likely include the co-development of a referral protocol outlining clear referral criteria, practitioner guidance and structured signposting (53). To further inform this work, we will conduct individual interviews with survivors (n=10) and shadow helpline phone calls (n=10) in collaboration with our partner Hourglass. These will focus on identifying the structural and practical

challenges older women face when accessing support, and on developing strategies to help address and overcome them.

We will conduct two focus groups involving a diverse range of stakeholders, including GPs, nurses, psychiatrists, clinical psychologists, social workers, DA practitioners, third sector representatives, and survivors (n = 7- 8 participants per group; total n = 15), to explore: 1) the suitability of current referral pathways, with a focus on identifying barriers and facilitators that affect access to support for older women, and 2) the development of new referral pathways including practical guidance for use in specific settings. We will ask professionals and survivors how A-PATH can fit into existing services and how collaboration between primary care and community-based organisations can be strengthened (54). Prior to our feasibility trial we will examine referral routes into A-PATH to ensure they are accessible.

Data will be analysed thematically using both deductive approaches – based on pre-specified research questions – and inductive analysis to capture insights emerging from participants' accounts (51, 55). Emerging themes will be reviewed and validated by our LEAP experts. To support meaningful participation and relationship-building, we will allocate time early in the project to develop trust through community engagement strategies. This will include working with health ambassadors and community research champions who can act as trusted intermediaries, helping to foster relationships, and ensure the research is accessible, and inclusive (56, 57).

Work Stream 3

Feasibility and acceptability study of A-PATH

We will pilot A-PATH using an individually RCT design (58), in which eligible older women will be randomly assigned to receive either the adapted PATH intervention (plus DA advocacy) or usual care (DA advocacy only). We will assess the suitability of outcome measures, identify an appropriate primary outcome, and explore the acceptability of randomisation for older women affected by DA.

The primary aim is to assess whether a fully powered large-scale study is feasible and acceptable, by collecting information about the recruitment and trial retention rates that may affect the main trial. We will establish recruitment pathways, and identify facilitators and barriers to recruitment, including whether specific formats of the intervention (e.g. online or materials written in English) present a barrier to older women. Estimation of the (study) retention rate will enable us to assess whether a main trial is feasible, and to adjust for withdrawal appropriately. We will evaluate the feasibility of collecting outcome data, and explore participant burden, length of time and difficulty in completing measures and missing data. We will consider how best to balance and stratify our randomisation (e.g., by age, co-habitation status, and site), with our sites chosen to increase diversity and serve both rural and urban areas.

Target population and inclusion/exclusion criteria

We will recruit women aged 60 years or older who have experienced or continue to experience DA. Our study will focus specifically on intimate partner violence in later life, rather than broader forms of DA such as abuse by adult children or other family members. Our inclusion criteria will be as follows:

- women aged 60 years or older

- who have experienced domestic abuse, including physical, emotional, psychological, sexual, or financial abuse, either currently or in the past
- are able to provide informed consent to participate in the study
- have sufficient cognitive capacity to engage in the intervention and complete study assessments.

Although we will try to be as inclusive as possible, our PPI pre-grant advisory panel have advised us that we will need to exclude women with a psychotic illness, those living with a severe drug or alcohol problem, or those living with dementia. Individuals currently engaged in other structured psychotherapeutic treatments will also be excluded. We will only recruit older women, given evidence that they are more likely to experience repeated and severe forms of abuse compared to older male victims (59). We will purposively recruit older survivors from linguistically and culturally diverse backgrounds (e.g., South and West London), with participating services drawn from varied geographical areas across England and Wales. These sites reflect differing levels of prior engagement with older survivors, enhancing the diversity of perspectives and increasing the generalisability of our findings. For the purposes of adapting PATH, we define 'older women' as those aged 60 and above—a threshold that reflects the distinct social, psychological, and health-related experiences associated with later life, and aligns with best practice in DA advocacy for older survivors (18, 19).

Sample size calculation

As this is a feasibility study, the primary aim is not to detect a statistically significant effect but to estimate key parameters necessary for designing a future large scale trial, including acceptability of the intervention, and sensitivity to change of outcome measures (60, 61). Therefore, a formal power calculation is not appropriate at this stage. We will randomly allocate thirty-eight survivors to DA advocacy (usual care), and thirty-eight to DA advocacy plus A-PATH (n = 76). Our proposed sample size is based on recommended guidance for feasibility trials (58, 61), allowing sufficient precision to estimate feasibility outcomes, and gather preliminary data on outcome measures (e.g., mental health, wellbeing, service use). Findings from this feasibility trial will inform robust sample size calculations for a future large-scale RCT by providing estimates of effect size variability and other key design trial parameters.

The sample size of 76 that we have chosen allows us to estimate a retention rate of 75% at 8 months to within a 95% confidence interval of ±10%. This sample size will also enable us to estimate the standard deviation of our candidate primary outcome (Clinical Outcomes in Routine Evaluation–Outcome Measure (CORE-OM); psychological distress; (62, 63)) in our target population with sufficient precision (64). Based on existing data, we expect the standard deviation of CORE-OM to be approximately 5 (63), which we will estimate within a 95% confidence interval of ±0.9. The standard deviation estimate will inform the sample size calculation for our future larger trial. These sample size calculations were performed in Stata v18. These retention rates align with those observed in previous trials identified in our review, demonstrating their feasibility and achievability (29, 65). Outcome measures will be administered at baseline (prior to randomisation), and at 4 and 8 months

(primary outcome timepoint) post-randomisation to assess changes over time and inform follow-up feasibility.

Randomisation will be conducted by an unblinded member of the research team using a secure online randomisation system (64). In cases where a practitioner or researcher identifies a significant risk of self-harm, suicide, or harm from others, unblinding may be required to ensure appropriate and timely support for the participant's safety and well-being. To ensure the safety of both participants and the research team, we will develop and implement comprehensive risk management protocols, which we will adapt from our previous and ongoing studies (e.g. REPROVIDE (66)). Two distinct protocols will be established: one to guide responses to participant distress during or after study activities, and another to address safeguarding concerns related to ongoing abuse or risk of harm. These protocols will be informed by best practice guidelines and developed in consultation with our clinical (CH) and safeguarding (AW) leads to ensure a trauma-informed and ethically robust approach throughout the study.

Primary outcomes

Feasibility of progressing to a multi-centre RCT will be assessed primarily through recruitment and retention of participants, as well as acceptability and absence of any major concerns such as multiple serious adverse events. Satisfaction on these key points will indicate the practicality of scaling up the intervention.

Primary feasibility outcomes:

We will collect data on: a) the number of referrals and proportion of women recruited and randomised to the study out of those referred, b) the proportion of those retained at 4 and 8 months, and c) the extent to which participants engage with the adapted PATH intervention as intended. Quantitative metrics which will include the percentage retained at primary endpoint (8 months) will be analysed alongside qualitative insights gathered through interviews with participants (both older women and practitioners) who withdraw or are lost to follow-up. This mixed-methods approach will enable us to assess the feasibility of achieving adequate retention and inform appropriate strategies to optimise it in a future full-scale RCT. Our stop/go progression criteria for retention (60) (baseline to primary outcome completion at 8-month follow-up out of number randomised) will be as follows:

a) trial retention rate approaching 80% will be considered supportive of feasibility for progression to a full trial
b) trial retention between 65% - < 80% indicative that a full trial is feasible, provided reasons for poor retention are identified and addressed, and
c) trial retention rates below 65% as indicative that our feasibility study is unlikely to progress to a full trial.

Intervention adherence will be assessed through session attendance, completion of core intervention components, and insights from qualitative follow-up interviews. We will explore reasons for non-attendance, with particular attention to barriers experienced by older women, to inform adaptations to the PATH intervention that enhance accessibility, engagement, and relevance for this population. As part of tailoring the PATH intervention to meet the specific needs of older women, we will adapt an existing fidelity framework to ensure core therapeutic components are delivered consistently while allowing appropriate flexibility for individual needs and

context (30, 31). This adapted framework will define essential elements of the intervention, identify adaptations needed (e.g. delivery mode, pacing, language), and include mechanisms for monitoring fidelity through practitioner training, supervision, and session checklists. This approach will help maintain intervention integrity while supporting relevance and responsiveness to older women's diverse experiences and preferences (35).

Secondary outcomes

As part of our feasibility RCT we will try to identify the most appropriate primary outcome measure for our large-scale multi-centre evaluation trial. We will estimate potential effectiveness on a range of candidate primary outcome measures, and their standard deviations (SDs). These will include:

a) psychological distress - Clinical Outcomes in Routine Evaluation–Outcome Measure (CORE-OM) (62); The 34-item CORE-OM assesses global psychological distress and is validated for sensitivity to change, test–retest reliability and UK normative comparison. Originally developed to evaluate the effectiveness of psychological therapies from several disciplines (67), it will be used in this study both as a continuous outcome (mean score) and a binary variable, using the clinical cut-off score of ≥ 9.9 (62).

b) depressive symptoms - Patient Health Questionnaire (PHQ) (68); The nine-item PHQ assesses depressive symptoms and demonstrates strong validity, reliability and sensitivity to change in older adults (69). In this study, it will be used as both a continuous measure (mean score) and a binary outcome based on the established clinical cut-off score of ≥10, indicative of probable major depression (69).

c) post-traumatic stress symptoms – Posttraumatic Stress Disorder Symptoms Scale (70); this 17-item questionnaire measures post-traumatic stress symptoms with a score ≥ 17 consistent with a clinical diagnosis of PTSD and has established validity in older samples (71), and

d) health-related quality of life - EQ-5D-5L (72); the EQ-5D-5L is a widely validated, standardised instrument that captures core dimensions of health, making it suitable for use across a range of populations and health conditions.

A ‘shortlist’ of potential primary outcome measures will be developed working closely with our LEAP group. We will explore participants’ and practitioners’ views of their suitability in the qualitative follow-up interviews. Participants will also be asked to record key demographic data at baseline and complete a brief feedback form when they finish the A-PATH sessions.

We consulted with lived experience experts to help us determine our primary outcome timepoint. Their feedback emphasised the importance of allowing enough time to observe meaningful and sustainable change. This input supported the selection of the 8-month timepoint as our primary outcome timepoint, ensuring participants have ample opportunity to complete A-PATH and to apply its content in their daily lives. This was considered especially important given the complexity of DA in later life. An 8-month follow-up allows us to capture longer-term indicators of impact and is supported by other DA intervention RCTs (15, 25, 30), and NIHR feasibility study guidance (41). While the feasibility study will include an 8-month follow-up, the full-scale trial will adopt a 12-month follow-up as the primary outcome timepoint, consistent with our approach in the PROVIDE trial (30, 31).

Health-economic analysis

We will explore feasibility of collecting economic data, and establish the optimum way of evaluating cost-effectiveness in a multi-centre evaluation trial. Our health economic evaluation will consider the perspectives of health and social care, public sector and society, using Quality-Adjusted Life Years derived from the EQ-5D (72) and a modified Client Service Receipt Inventory (73). We will tailor our resource use questionnaire, in consultation with our PPI group and refine it through early testing. We will explore the completeness, acceptability, and relevance of these measures, including the feasibility of collecting data from both NHS and wider service perspectives.

Process evaluation

We will use realist methodology to identify what works, for whom and in what circumstances (74), utilising the Medical Research Council (MRC) process evaluation framework (75). A key focus of the process evaluation will be to explore the acceptability of the A-PATH intervention, including how it is experienced by participants and practitioners within real-world settings. We will conduct qualitative process interviews with up to 15 survivors who participated in the A-PATH intervention, capturing a range of engagement levels, alongside interviews with up to 10 survivors from the usual care group to explore comparative experiences. Where possible, we will also conduct short informal 'exit' interviews (participants who drop out of the intervention or study), to better understand reasons for disengagement. We will interview DA practitioners (n = 6) to gather insights into what elements of the intervention are working well, and what needs to improve, and to inform our understanding of A-PATH's mechanisms of change (31).

The process evaluation work will enable us to identify candidate factors that may require adaptation within the PATH protocol, thereby informing refinements aimed at improving retention among older women in a future large trial. We will explore DA practitioners' perspectives on factors that facilitate or hinder engagement among diverse groups of older women and how obstacles can be overcome (76). We will use this data to inform the refinement of our logic model to be fully tested in a future large-scale RCT (43). A comprehensive flow chart detailing all stages of the research is presented in page 20. This final Work Stream will synthesise the study's key outputs, including the adapted intervention, an updated logic model, a co-produced referral pathway, and the delivery of a feasibility pilot RCT.

Health technologies being assessed

A-PATH Intervention

Women randomised to A-PATH will be offered up to eight weekly or fortnightly sessions, followed by two booster sessions, 1 month and then 3 months post-intervention. The schedule will be flexible and revised according to participant preferences. A-PATH is designed to support women in recognising, and exploring psychological consequences related to experiences of DA, while developing coping strategies to manage conflict and other stressors (30, 31). In collaboration with survivors and practitioners we have identified several adapted topics relevant to older women, including "What I Want to Happen" and "Planning for the Future" which will be incorporated into the intervention content. The therapeutic sessions will be

delivered alongside standard DA advocacy support. When adapting PATH, we will draw on the '*psychosocial readiness mode*l' of change (77), which highlights awareness, perceived support and perceived power as key factors influencing behaviour change in women affected by DA. As part of this work, we will produce a revised logic model with specific behaviour change components for this group.

Recognising the diverse barriers faced by older women including digital exclusion, mobility limitations, frailty, and social isolation—the intervention will employ a flexible delivery model that accommodates individual needs and preferences. Participants will be offered flexible access to the intervention through multiple formats—face-to-face sessions in accessible community or healthcare settings, telephone-based therapy, or online video calls—ensuring it is both feasible and tailored to individual preferences. Insights from our lived experiences study indicated that this flexibility ensures that women without internet access or digital skills (e.g. those aged over 80; (78)), can still engage fully, while those comfortable with technology have the option of remote support. All DA practitioners will receive regular supervision by RAD (25), throughout the study period.

Usual care

Women in the control group will receive usual care in the form of DA advocacy support but will not participate in A-PATH sessions. The advocacy support will include a package of legal, housing, financial, and safety advice, provided by our partner Hourglass, a leading national charity and experts in DA of older women (8). This support will be provided by trained DA practitioners to ensure women receive accurate, timely, and trauma-informed guidance. The package is designed to empower survivors with critical knowledge about their rights, options for financial independence, and personalised safety strategies, and support women in making informed decisions about their future. Advocates in this group will not receive specialist training in psychological methods.

What might the A-PATH intervention look like*?

A-PATH is designed to be **flexible** and **client-led**. In the first session, the practitioner and client explore the psychological impact of abuse by focusing on **validating lived experiences.** Subsequent sessions are collaboratively selected by the client and practitioner, ensuring A-PATH remains responsive and person-centred. Core themes include **strengthening self-esteem**, **promoting self-care**, **reducing trauma and psychological distress**, and **building emotional resilience** to support recovery and wellbeing. The final session focuses on reviewing progress, reinforcing key strategies, and **planning for the future.**

** Informed by the original PATH trial, our qualitative research, lived experiences, and wider evidence on tailoring DA interventions for older women (15, 25)*

Knowledge mobilisation, dissemination and impact

Knowledge mobilisation

Despite progress in DA research and policy, older women survivors continue to be marginalised, and experience social, economic, emotional, and institutional exclusion (19). Our aim is to translate research into meaningful action by:

a) building an interactive A-PATH website, with survivors and practitioners, to share findings in accessible, engaging formats and actively invite feedback from a broad public and professional audience;
b) collaboratively developing a set of evidence-informed referral pathways for use across the NHS, local authorities, and the third sector;
c) facilitating co-designed workshops and roundtable discussions with survivors, frontline professionals, and community organisations to reflect on our findings and collaboratively shape the design of a future large-scale trial.

We will follow guidance on community-based knowledge mobilisation in DA research (79, 80). Central to our approach is the development of equitable partnerships with survivors, practitioners, and third-sector organisations, recognising their essential roles in shaping, delivering, and sustaining meaningful change.

Dissemination

Working with key stakeholders, and lived experience experts we will co-develop a dissemination plan to identify who we need to engage, how to reach them and ensure outputs are accessible and inclusive. We will draw on our National Peer Research Network for Preventing Domestic Abuse in Older People, hosted by UCL, which brings together survivors, researchers, practitioners, and community organisations across the UK. The network will help us reach a broad and diverse audience including health and social care professionals and third-sector organisations. Our dissemination activities will be facilitated with the support of our LEAP experts and third sector partners. We will publish our trial findings in peer-reviewed academic journals and present these in national DA conferences. We will attend local community events attended by commissioners of DA services to present our results and host a public knowledge exchange event to share our findings and our 'good practice' referral pathway.

Impact

This feasibility trial will provide critical preparatory work for addressing a major gap in supporting older women affected by DA. We anticipate direct impact on improving awareness of DA affecting older women, and guidance on suitable referral pathways including videos describing our key findings. By adapting the evidence-based PATH intervention to meet the specific needs of older women, this study will improve its future accessibility by tailoring its content to the unique experiences, health profiles and barriers faced by older women. This study will also strengthen the current evidence base by generating essential data on recruitment, retention, and outcome measurement in this population. Developing an intervention that is shaped by lived experience and is responsive to the psychological, social and practical needs of older women allows us to amplify the voices of older survivors, ensuring we lay the foundations for scalable change in DA support services and practice across health and third-sector settings. If A-PATH is feasible and acceptable, it will lead to a larger HTA trial evaluating the first evidence-based trauma-informed and age-responsive psychological intervention for older women affected by DA.

Project management

Timeline, and milestones of the project

The project will be delivered over 30 months. The first 8 months will focus on adapting the PATH intervention and co-developing setting-specific referral pathways. This will be followed by a 16-month feasibility study, including an 8-month recruitment phase and an 8-month primary outcome assessment period. In the final 6 months we will complete data analysis, interpretation, and dissemination through reports, academic publications, and stakeholder engagement.

Work Stream 0 (Months -3–0): We will obtain all necessary NHS Ethics and Health Research Authority (HRA) approvals between October and December 2025—three months before the planned study start date—to ensure full regulatory compliance and allow adequate time for site setup and preparatory activities.

Work Stream 1 (Months 1–4): We will conduct two participatory focus groups and a roundtable discussion with key stakeholders to inform the adaptation of the PATH intervention. During this phase, we will also revise the original training package for DA practitioners to ensure it is relevant for supporting older women. During this WS we will field-test the adapted PATH intervention to gather preliminary data on barriers and facilitators in real-world settings.

Work Stream 2 (Months 5–8): We will conduct individual interviews with survivors and focus groups with professionals to explore and optimise setting-specific referral protocols across services such as general practice, emergency care, acute wards, and community settings.

Work Stream 3 (Months 9–30): A 16-month feasibility study will be undertaken (Months 9–24), including a 8-month recruitment phase (Months 9–16), supported by our established National Network, third sector partners and participating NHS Trusts. Training for DA practitioners will take place before recruitment begins to ensure preparedness across sites. Quantitative and qualitative data will be analysed and disseminated through academic publications (Months 25-30), conference presentations and community stakeholder workshops.

Recruitment centres and partners

Building connections to support recruitment. Recruitment of older women affected by DA requires building trusted connections with services and communities that support this population. We will engage with NHS settings (e.g. general practice, and mental health services), third-sector organisations and community groups working with older adults. These partnerships will help us identify potential participants sensitively and ethically, while ensuring outreach is inclusive of women from diverse backgrounds, drawing upon established IRIS (Identification and Referral to Improve Safety) sites (34) and our existing networks. Our feasibility trial will be delivered across **four principal recruitment centres**—London, Bristol, Swansea, and North Devon—**each supported by two affiliated local recruitment sites (NHS and community-based),** all with established research infrastructure.

- **London (University College London; 4 sites; projected recruitment 32 participants)**: UCL brings extensive experience in leading complex mental health and DA trials. It has well-established NHS and community sector partnerships to support recruitment and delivery, and led our lived experiences study. This centre will recruit from the North West London NHS Foundation Trust, Camden and Islington NHS Trust (and partner Trusts through Noclor), and local centres (Age UK, North London Women's Aid and SafeLives), contributing a total of 4 individual sites.

- **Bristol (University of Bristol; 2 sites; projected recruitment 16 participants)**: University of Bristol is a national and international leader in DA research. The team at Bristol has longstanding collaborations with local services and a proven track record of delivering intervention studies with women affected by DA. The Bristol team will recruit from NHS Bristol, North Somerset and South Gloucestershire Trust, and local DA services that participate in ongoing trials (Bristol Next Link).
- **Swansea (Dewis Choice; 2 sites; projected recruitment 14 participants)**: Dewis Choice is the only UK initiative specifically designed for older survivors of DA, with expertise in age-informed, trauma-sensitive practice. This site will work closely with the Centre for Age, Gender and Social Justice, Solace Women's Aid and the Swansea Bay University Health Board.
- **North Devon (North Devon Against Domestic Abuse – NDADA; 2 sites; projected recruitment 14 participants)**: As a trusted local service with embedded links to health and social care providers, NDADA offers crucial insight into rural service delivery for older women affected by DA in underserved areas. This site will receive referrals directly from NDADA and the Royal Devon University Healthcare NHS Foundation Trust.

Our four recruitment centres provide geographic and demographic diversity, supported by strong community and clinical networks across ten active recruitment sites. Based on empirical benchmarks from NIHR feasibility studies (81, 82), we anticipate a recruitment rate of one participant per site per month. Across ten sites over 8 months—the recommended window for engaging underserved groups—this provides a potential total of 80 participants, allowing a buffer above our target of 76 to mitigate potential recruitment challenges. This estimate reflects realistic feasibility assumptions (61) and is enabled by our plans to include national recruitment pathways e.g. via Hourglass helplines and an online delivery option.

Third sector partners

To ensure inclusive recruitment, we will collaborate with key third sector organisations that have established expertise in supporting older women affected by DA. These include **Hourglass**, the only UK-wide charity dedicated to ending abuse and neglect of older people; **Women's Aid**, a national federation supporting women experiencing DA; **SafeLives**, a leading organisation with cross-sector frontline expertise; **Age UK**, which delivers safeguarding and support services for older adults; and **Dewis Choice**, a pioneering Welsh initiative co-produced with older survivors and led by one of our co-applicants. We will also recruit through **North Devon Against Domestic Abuse (NDADA**), a specialist provider of refuge and community-based support for women in rural and underserved areas. These trusted partners will support participant identification, DA practitioner recruitment, and help develop trauma-informed, age-sensitive engagement strategies. All materials will be co-designed with our LEAP group to ensure they are accessible, respectful, and reflective of older women's experiences. The longstanding credibility and community presence of our partners will be central to improving reach, trust, and engagement with underserved populations.

NHS Trust partners

Recruitment through NHS Trusts will be a key component of our feasibility trial. We will work closely with general practice teams including mental health services, community mental health older adult teams, safeguarding leads, and health-based independent domestic violence advisors (IDVAs). These professionals are well placed to identify older women who may be experiencing DA but are not currently accessing specialist support. Site-specific research champions from our National Network will promote the study among frontline staff.

Recruitment of DA Practitioners

We have already identified 1 practitioner (DA champion) in each of our sites. The remaining practitioners will be identified through established DA support networks, our partner Hourglass, specialist services, and community organisations and will be led by our safeguarding lead (AW) and our DA champions. We will adopt a proactive, evidence-based recruitment strategy aligned with our previous trial (30), and the landmark IRIS study (54), where each study site has a designated advocate coordinating referrals. Our community-led approach—centred on collaboration with survivors, frontline professionals, and voluntary sector leaders—places us in a strong position to deliver this research effectively. Community outreach will be a key component of our engagement strategy (83), supported by our National peer research network preventing domestic abuse in older people initiative.

Study management, and expertise in the study team

Our multidisciplinary team brings together expertise from psychology, social science, methodology, statistics, general practice, nursing, DA advocacy and lived experience. Collectively, we have a strong track record in the development and evaluation of complex, community-based interventions for older adults and women affected by DA. Our team includes Dr Vasiliki Orgeta, a specialist in psychological interventions in later life (29), who co-led the recent Lancet Commission on Trauma and Ageing, which highlighted the underrepresentation of older women affected by DA in both research and policy (84). Dr Helen Cramer is a nationally recognised leader in perpetrator-focused interventions (57, 66). They will receive institutional support from both University College London and the University of Bristol—centres of excellence in older adults' mental health and DA research.

Dr Roxane Agnew-Davies is a clinical psychologist with over 30 years of experience in specialist mental health services for women affected by DA. She led the development of PATH implemented in various sites across England and Wales and has published widely on DA. Dr Alison Gregory is a nationally recognised expert on the secondary trauma experienced by informal supporters of DA survivors. She has led multiple impactful research projects with strong partnerships with Women's Aid, local authorities, and public policy stakeholders to translate research into practice.

Dr Victoria Vickerstaff brings expertise in statistical methodology and applied health research, with a robust track record of leading the design and analysis of several multidisciplinary NIHR-funded trials in older people and palliative care settings. Amanda Warburton-Wynn is the Domestic Abuse Health Strategic Lead for Cambridgeshire County Council, working with health agencies across the county to improve the recognition and response to domestic abuse. She developed the Older People's DASH risk assessment tool for DA that has been used in Cambridgeshire

since 2021 and is currently being piloted in Norfolk and Wales. She is also an independent researcher, specialising in improving support for older people and people with disabilities who are subjected to DA and sexual violence.

Marion Goodchild brings over 25 years of experience in nursing with a strong track record of championing patient-centred approaches in health and social care research, such as enhanced training for cancer care teams in identifying DA. She has contributed to multiple research programmes as a public advisor, helping to improve participant engagement and inclusive practice. Dr Claire Hawcroft is a practising GP and clinical academic with expertise in the design and evaluation of trauma-informed interventions for women affected by DA across primary care and community settings. Professor Sarah Wydall is a leading expert on DA in later life. As Principal Investigator of the Dewis Choice project, she has pioneered multi-agency, participatory responses for older survivors, including those with dementia, and developed innovative tools and frameworks now used nationally.

Data management, reporting and analysis

Questionnaire and clinical data will be collected and stored securely in REDCap, with access restricted to authorised research team members. Administrative data—such as participant names, contact details, and follow-up information—will be pseudonymised and stored in a separate, password-protected database (e.g. Microsoft Access), in accordance with data protection regulations. This separation of identifiable and research data ensures confidentiality, minimises risk of data breaches, and complies with Good Clinical Practice (GCP). All personal data, and data handling, processing, storage, and destruction will comply with the Data Protection Act 2018 and GDPR regulations. Data quality assurance procedures will be in place to verify the accuracy of information entered from any hard-copy questionnaires. Recruitment and retention data will be summarised using a CONSORT flow diagram. A statistical and health economic analysis plan for the feasibility trial will be developed by the trial statistician (VV) in collaboration with the Trial Management Group.

Study oversight and safety protocol

Oversight will be provided by our LEAP panel, which will meet quarterly to inform and contribute to all Work Streams. The Programme Management Group (PMG) will meet monthly throughout the study to oversee delivery, with a particular focus on monitoring recruitment against projected targets. Meetings will include regular review of site-level performance, identification of emerging challenges, and implementation of corrective actions where necessary to ensure timely and effective recruitment. Additional ad hoc meetings will be convened should any significant issues arise.

The quality and integrity of trial data will be monitored by an independent Data Monitoring and Ethics Committee (DMEC) and a Trial Steering Committee (TSC). A comprehensive safety protocol, aligned with WHO guidelines (85), will be used to record and monitor adverse events. The TSC will review any reported harms associated with the A-PATH intervention. All serious adverse events will be reported immediately to the sponsor, TSC, and the NHS Research Ethics Committee (REC) in accordance with the UK Policy Framework for Health and Social Care Research (2020). Participant safety will be overseen by our designated Safeguarding Lead

(AW), who will conduct monthly safety reviews and report findings to both the PMG and LEAP.

If participants (older women or practitioners) become distressed during any sessions or assessments, the researcher/DA practitioner will respond in line with established safeguarding and distress protocols. Immediate emotional support will be offered, and the session paused or discontinued if needed. With the participant's consent, a referral will be made to appropriate support services, including local DA specialists and mental health providers. All research staff will be trained in trauma-informed approaches and will follow a clear distress and a separate safeguarding protocol approved by the ethics committee. Our study is informed by best practice guidelines from UK third sector organisations, including SafeLives, Women's Aid, and AVA (Against Violence and Abuse), emphasising the importance of conducting research that is trauma-informed, survivor-centred, and grounded in principles of safety and confidentiality. Researchers will be trained to recognise distress, and prioritise participant autonomy and well-being. All procedures will align with the ethical standards outlined by the UK Research Integrity Office (UKRIO) and the HRA guidance for research involving vulnerable adults, with input from our lead specialist DA advisor (AW) during our protocol development.

Equality, diversity, and inclusion

We have considerable expertise in recruiting participants from diverse communities. We will engage with seldom heard populations (e.g., older women from Black, Asian and minoritised ethnic communities, those that identify as LGBTQIA+), which face unique barriers in coping with abuse (76, 86). To encourage recruitment from non-English speaking communities, we will implement inclusive and culturally sensitive engagement strategies throughout the study. We will work with established leaders (e.g., Hourglass, Women's Aid), to ensure wording, images, and recruitment methods are as inclusive as possible. Recruitment materials (i.e., participant information sheets, posters, briefings to teams) will be reviewed for cultural appropriateness. We will work in partnership with community organisations that have established trust within ethnic minority communities to raise awareness of the study. We will work closely with local services and faith groups that support minoritised communities, to increase our insight of how best to serve these communities. We will collect data on individuals who were potentially eligible but excluded due to limited English proficiency, to inform inclusive recruitment strategies for a full-scale RCT, ultimately improving the generalisability of the adapted PATH intervention.

Intellectual property and commercialisation

All novel materials developed during this project will be identified and documented as potential intellectual property (IP). We will work closely with UCL's IP team to develop our IP strategy, and ensure that any outputs maximise public benefit while maintaining accessibility for organisations who support older women affected by DA. The overarching aim is to ensure that A-PATH delivers lasting value — improving outcomes for older survivors, and equips third-sector organisations with the tools to support their recovery and well-being**.** We will explore opportunities to expand the programme's reach through partnerships with third-sector organisations, licensing arrangements, or social enterprise models.

Logic model of A-PATH - Adapting Psychological Advocacy Towards Healing for older women affected by domestic abuse

Population

Population:
Women aged 60 years or more

Physical health risks
- Increased risk of frailty
- Increased incidence of chronic conditions

Psychological harm
- Depression, anxiety and trauma symptoms
- Shame & self-blame

Social risks
- Isolation
- Stigma
- Lack of support

Financial harm
- Economic abuse

Systemic risks
- Ageism in services
- Limited access to support

Theory, Assumptions & Implications

Empowerment Theory
- Building autonomy
- Enhancing self-efficacy
- Challenging systemic barriers

Assumptions & implications
- Gaining control
- Building confidence
- Promoting healthy relationships
- Empowerment through access to knowledge
- Autonomy and voice

Approach
- Focusing on strengths
- Avoiding paternalism
- Clear & accessible information
- Challenging ageism, stigma and exclusion

Intervention

A-PATH intervention
- Uses the psychosocial readiness model & CBT principles
- Strategies to cope with distress
- Enhances self-esteem
- Strengthens resilience
- Promotes self-care

Mechanisms of change
- Awareness
- Perceived support
- Perceived power

Control group
- Receives usual DA advocacy

DA advocacy
- Practical, legal, financial & safety advice
- Delivered by third sector and local DA services

Short Term outcomes

Psychological well-being:
- Improved self-esteem
- Reduced psychological distress

Sense of control
- Increased confidence
- Increased self-efficacy
- Greater control over routines & choices

Social connection
- Reduced social isolation
- Improved trust in others
- Rebuilding relationships

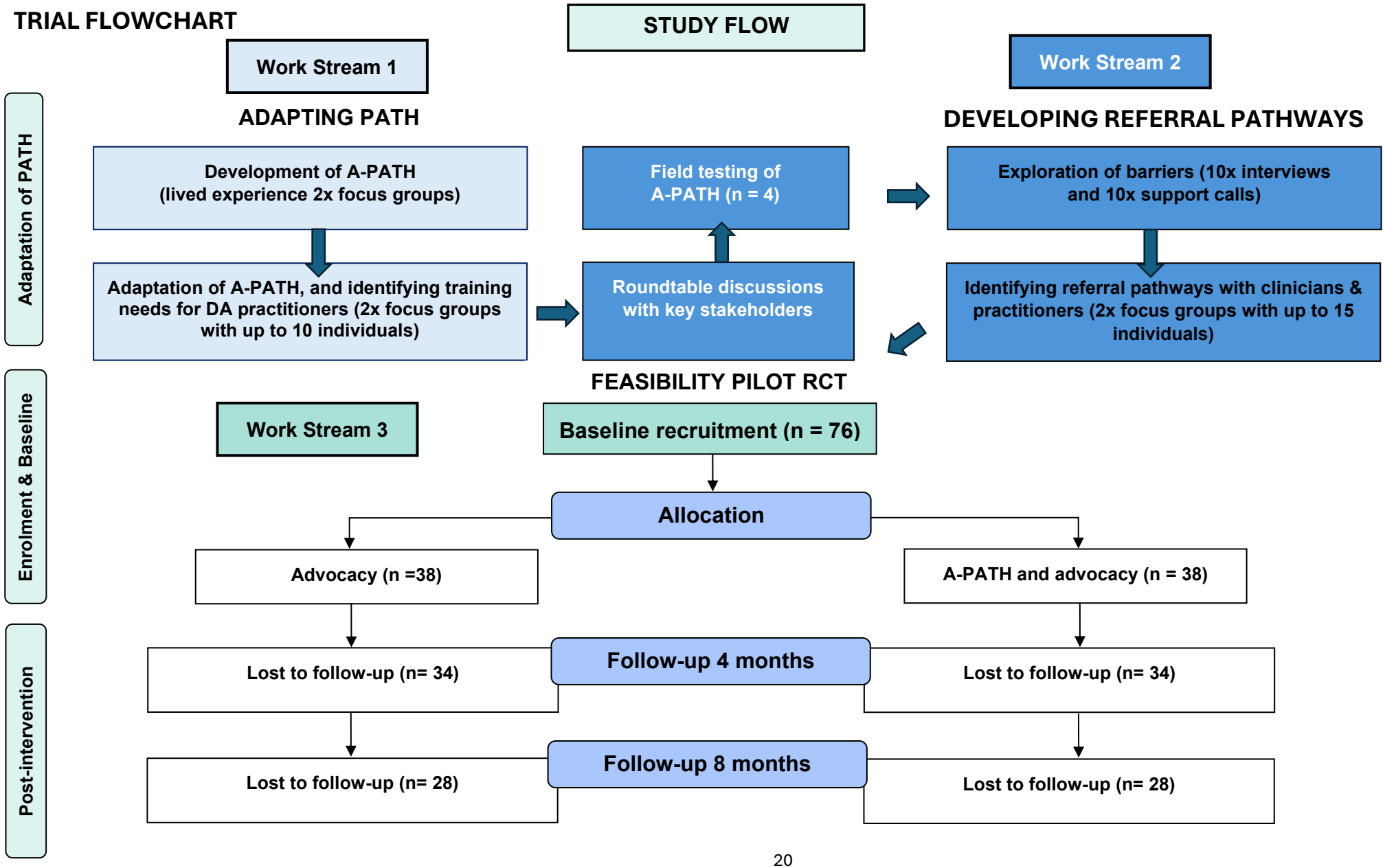
Long Term Outcomes

Psychological well-being:
- Reduce depression
- Reduce trauma symptoms
- Living with dignity
- Thriving beyond safety
- Improved well-being
- Strengthening resilience

Safety
- Awareness of risk
- Reduction in exposure to harm

Services engagement
- Increased awareness of services
- Improving ability to navigate services

Structural change
- Inclusive care systems
- Tackling ageism and stigma


TRIAL FLOWCHART
STUDY FLOW
Work Stream 1
Work Stream 2
ADAPTING PATH
DEVELOPING REFERRAL PATHWAYS
Adaptation of PATH
Development of A-PATH
(lived experience 2x focus groups)
Field testing of
A-PATH (n = 4)
Exploration of barriers (10x interviews
and 10x support calls)
Adaptation of A-PATH, and identifying training
needs for DA practitioners (2x focus groups
with up to 10 individuals)
Roundtable discussions
with key stakeholders
Identifying referral pathways with clinicians &
practitioners (2x focus groups with up to 15
individuals)
FEASIBILITY PILOT RCT
Enrolment & Baseline
Work Stream 3
Baseline recruitment (n = 76)
Allocation
Advocacy (n =38)
A-PATH and advocacy (n = 38)
Post-intervention
Follow-up 4 months
Lost to follow-up (n= 34)
Lost to follow-up (n= 34)
Follow-up 8 months
Lost to follow-up (n= 28)
Lost to follow-up (n= 28)

A-PATH: Adapting Psychological Advocacy Towards Healing for older women affected by domestic abuse

Background and rationale

Data published by Age UK in 2023 indicate that more than 375,000 older women aged 60 to 74 experience domestic abuse (DA) in England and Wales (1). The 2025 Femicide Census has revealed that one in eight women killed by men in the past 15 years was aged over 70 (2). DA can lead to serious physical health injuries for older women (3), often associated with prolonged recovery periods and an increased risk of mortality (4, 5). DA has profound emotional and psychological impacts, including increased severity of anxiety and depression (4), which significantly diminish older women's well-being and quality of life (6). Despite a growing number of older women experiencing DA, reports published by our partner Hourglass (7, 8) highlight persistent gaps in support and mental health provision for older women (9).

DA can expose older women to a range of harmful behaviours that are frequently underreported and poorly recognised within services (10, 11). These may include physical and emotional abuse – such as sustained verbal belittlement and psychological manipulation – as well as sexual abuse involving coercive or non-consensual acts, often occurring within long-term relationships (12, 13). Financial abuse is also common, typically involving the misuse of pensions, or the restriction of access to personal funds (14). Controlling behaviours – such as limiting access to healthcare or isolating individuals from social networks – further undermine older women's autonomy and well-being (14). These forms of abuse are often exacerbated by generational norms, such as the expectation that older women should endure hardship, and by systemic barriers, such as digital exclusion and insufficient training among professionals, which together limit access to timely support and care (15, 16).

The unique barriers faced by older women affected by DA are frequently overlooked in mainstream service provision and academic research (17). Abuse may span decades, exposing women to cumulative trauma, and become embedded in long-term relationships, making it harder to recognise or disclose (18). Many women may not recognise controlling behaviour or financial coercion by a long-term partner as abuse, particularly when such patterns have been normalised over time (19). For some older women, disclosing abuse may carry a significant risk of losing their home, financial security, or contact with grandchildren (20). Living with a chronic condition or frailty, increase dependence on the perpetrator, thereby reducing opportunities for older women to seek help (4).

Health and social care professionals sometimes miss signs of abuse, often attributing injuries or distress to age-related conditions or frailty rather than recognising them as intentional harm (9, 21). Our qualitative study has shown that digital exclusion—caused by limited access to technology or digital literacy (22)—prevents many older women from accessing support, or engaging safely with remote interventions (18). These factors not only increase older women's vulnerability to sustained and repeated abuse, but also highlight the need for targeted support (23), so that older women can get the help they need (18).

Psychological interventions are a vital component of the care pathway for women experiencing DA (24, 25), addressing the significant mental health impacts such as

trauma, and depression (19, 26). While these interventions are typically accessed once immediate safety needs have been met (15, 27), there is growing evidence supporting their value earlier in the help-seeking journey, including for women who remain in abusive relationships but are beginning to engage with services (15). In these contexts, psychological support can help build emotional resilience, strengthen coping strategies, and support informed decision-making around safety and self-care (25).

Psychological interventions that are not tailored to meet the specific needs of older women are less likely to be useful to female survivors and may even be harmful (28). Such harm can include retraumatisation, feelings of alienation or invalidation, and reduced engagement with services perpetuating health inequalities faced by older women (20, 27). Our review reporting on the clinical effectiveness of psychological treatments for older people exposed to trauma, found no interventions for older women who have experienced DA (29). This project aims to address this critical service gap by adapting an existing evidence-based psychological intervention - Psychological Advocacy Towards Healing (PATH) - originally developed for women of childbearing age surviving abuse (30, 31), to meet the unique needs of older women.

Developed by Agnew-Davies and colleagues (32), PATH integrates cognitive behavioural therapy (CBT), experiential and feminist theoretical frameworks to equip women with practical skills for managing the psychological distress associated with abuse (31). Although the PATH intervention has demonstrated effectiveness in reducing psychological distress and depressive symptoms in women aged 18 to 67 (30), its suitability for older women remains unclear, particularly in addressing their distinct experiences and the specific barriers they face when seeking help (20). Our research has highlighted a range of unique and intersecting factors that make it harder for older women to access support and cope with abuse (9, 11). These include digital exclusion, frailty and social isolation (33), which are often exacerbated by ageism within health and social care services, and intensified by prolonged coercive control that restricts social networks (4). As part of this study, we will also seek to examine the need for and, as necessary, develop specific referral pathways for older women who experience DA. Improving access is an essential pre-requisite to more older women getting psychological support and more equitable access and service provision (25).

Although PATH promotes skills to enhance women's self-esteem, assertiveness, and resilience – core components of long-term recovery and empowerment (31), it lacks adaptation for age-related factors such as living with frailty and the complexities of lifelong abuse or late-onset abuse (13, 19). It does not address for example, barriers commonly faced by older women, such as digital exclusion, generational stigma around help-seeking, and dependence on abusers for care or housing (18, 19, 22). As a result, the standard PATH intervention may not be accessible, acceptable, or effective for this population without modification (34, 35). Adapting PATH to reflect the specific needs and circumstances of older women could significantly enhance its reach and relevance, extending the benefits of this evidence-based intervention to a significantly underserved group (31).

In focus group discussions with older women survivors, participants emphasised the value of accessing psychological therapies that can validate their experiences, and reported that they sometimes receive practical and emotional support through local statutory DA practitioners. Currently however, DA practitioners are not trained to deliver specialist psychological support, limiting the scope of care available to older women. In this project our aim is to train DA practitioners to deliver the adapted PATH intervention (A-PATH), enabling them to provide psychologically informed support alongside advocacy as usual (26).

Aims and objectives

Our overarching aim is to improve the mental health of older female survivors of DA, and support their well-being and recovery. To achieve this, we will: i) adapt the core components of the existing PATH intervention to ensure they are tailored to the needs and preferences of older women, (ii) strengthen and tailor referral pathways to enhance accessibility with support services, and, iii) determine the feasibility and acceptability of the adapted PATH intervention and trial design.

The objectives are to:

1. Adapt the PATH intervention specifically for older female survivors, including the development of appropriate referral pathways;
2. Identify the main variables that may influence the design of a full-scale trial, including factors affecting the evaluation of the cost effectiveness of this intervention;
3. Determine the acceptability and feasibility of the adapted PATH and trial design from the perspectives of clients and advocates and;
4. Explore the experiences and perceptions of those receiving and delivering the adapted intervention and its context.

Why is this research important in terms of improving the health and wellbeing of the public and health care services?

Older women affected by DA often access support through NHS services such as general practice (GP), emergency departments and hospital wards, where abuse may be disclosed or suspected during consultations related to physical or mental health concerns (36). The impact of DA in later life is often far-reaching, long-lasting and cumulative—exacerbating existing health conditions and contributing to the onset or worsening of mental health disorders (3, 6). Prolonged exposure to abuse can increase vulnerability to chronic health conditions, such as cardiovascular disease, musculoskeletal pain (37, 38) and elevates risk of frailty (39). Timely and tailored psychological interventions delivered by DA advocates have the potential not only to alleviate immediate psychological distress but also to reduce long-term health risks and subsequent service use among older women (40).

What is the evidence that the intervention is ready for HTA evaluation?

PATH is a manualised, trauma-informed intervention grounded in cognitive-behavioural principles, which is effective in reducing psychological distress and depression for women experiencing DA (25). This existing evidence base highlights the intervention's readiness for further adaptation and targeted evaluation, particularly for underserved groups such as older women (35). Preliminary work by

our team that included a systematic review of psychological interventions for older people exposed to trauma has identified a lack of tailored psychological interventions for older women affected by DA (29). Additionally, our Patient and Public Involvement (PPI) activities have consistently highlighted a significant gap in age-appropriate services and a pressing need for psychological support that recognises the unique experiences and barriers faced by older women.

Research questions

Main research question: Is it possible to adapt PATH in a way that is accessible to a diverse range of older women to empower and support their wellbeing and recovery?
1a) What does an adapted PATH intervention look like, and what types of formats and delivery are wanted?
1b) What are key stakeholders' views about current referral pathways?
1c) What are the barriers and facilitators for older women needing help?
1d) What solutions can be applied to optimise referral pathways for older women?
1e) Is it feasible to deliver A-PATH in primary care and community settings?
1f) What is the feasibility of collecting outcome measures and study procedures?
1g) What is the acceptability of A-PATH and trial conditions to older women and DA practitioners?
1h) How can the PATH intervention be adapted to effectively support the diverse needs of older women affected by DA including those from ethnic minority backgrounds and varied geographic settings?
1i) How can a revised fidelity framework ensure consistent and culturally responsive delivery across different contexts?

Methodology/Plan

The programme will be delivered over 36 months and will comprise 3 interlinked work streams (WSs). We will adapt PATH with the support and input of a Lived Experience Advisory Panel (LEAP). PPI will be embedded throughout all stages of the research, including through the LEAP panel. To ensure diversity and inclusivity, we will proactively recruit LEAP and PPI members through outreach specialist services that support older women from a range of ethnic, cultural and sexual identity backgrounds. One of our co-applicants brings lived experience of DA and will play a key role in project management, leading on PPI recruitment and co-facilitating the LEAP panel.

Design

We will use a mixed-methods design, a pilot randomised controlled trial incorporating acceptability and feasibility, and a process evaluation with embedded qualitative work (41) to:

a) WS1 - adapt the components and materials of PATH including practitioner training resources, and handouts for service users
b) WS2 - identify suitable and accessible referral pathways to facilitate older women's access to psychological support and advocacy services, and
c) WS3 - pilot test the feasibility and acceptability of A-PATH using a RCT design, alongside a nested qualitative sub-study to explore the acceptability,

experiences, and contextual factors influencing engagement with the intervention.

Work Stream 1

Part 1. *Adaptation and development of A-PATH*

In two focus group meetings (each with up to 8 survivors purposefully recruited to reflect diversity in age and ethnicity), we will explore the content of the intervention, elicit preferences for key topics (e.g., *Making sense of it all, Flashbacks and intrusive memories, Injured self-esteem*), and identify any gaps relevant to the needs of older women. The focus groups will be used to explore the current content and delivery of the PATH intervention, with the aim of understanding why it may not fully address the specific needs of older women (42). Discussions will identify specific elements of the intervention in relation to older women's experiences, as well as their preferences and barriers to accessing support. Prior qualitative findings and relevant literature (29) will be used to inform and enrich these discussions, ensuring they are grounded in existing evidence and lived experience.

While the PATH intervention has primarily been delivered to women who have left abusive relationships (30), we recognise that many older women remain in an abusive relationship due to complex factors such as dependency, care needs or housing barriers (18, 19). As part of this study, we will explore whether PATH is usable and acceptable for older women who are still in abusive relationships, and identify what adaptations may be necessary to ensure it is safe, appropriate, and responsive to their specific needs and circumstances. We will actively involve older women with lived experience in informing and shaping the adaptation of PATH in a meaningful and respectful way (42). When developing the intervention materials, we will draw on the linguistic and cultural expertise of our community partners to ensure accessibility and relevance for women whose first language is not English (43).

Our focus groups will be supported by our LEAP experts (44), and will invite older women survivors of DA to suggest improvements to PATH. The revised intervention will then be presented in a roundtable discussion with key stakeholders (representatives from Hourglass, Women's Aid, Age UK, DA practitioners; n = 8) to gather further feedback (35). This final stage will inform refinements to the content, materials, and logic model (see page 19). All components of the A-PATH intervention will be summarised using the Template for Intervention Development and Replication (TIDieR) checklist to support transparency and future scalability (45).

Preparation for the feasibility RCT

We will field-test our intervention with 4 survivors, to explore the practicalities of different delivery modes - telephone, online, and in-person - allowing us to identify any unaddressed barriers. In partnership with Solace Women's Aid (22), we will explore strategies to mitigate digital exclusion, which remains a significant barrier for older women. This could include for example offering non-digital options (e.g. phone, face-to-face, printed materials), providing basic digital skills support where appropriate, and working with community partners to ensure access to trusted spaces with digital resources. These strategies will be co-informed by the lived experience of older women to ensure accessibility, safety, and choice. We will also

explore practical factors influencing session attendance, such as mobility limitations, transport barriers and caring responsibilities (46).

Part 2. *Adaptation of training package for DA practitioners*

We will conduct two focus groups with DA practitioners (n=10), including those with and without experience working with older women, to assess the original PATH training package (30). To ensure the adapted PATH intervention is responsive to the needs of survivors from diverse ethnic, cultural, socioeconomic, and sexual identity backgrounds, we will also examine the inclusivity of the practitioner workforce, considering how diversity among practitioners can support more equitable and effective delivery. This includes exploring who delivers the intervention, how they are trained, and whether a more inclusive and representative workforce could enhance engagement with A-PATH. We will consider how practitioner background, cultural competence, and life experience influence delivery, and identify opportunities to strengthen training to support a more diverse, trauma-informed and inclusive approach.

Based on this input, we will develop and deliver a flexible training programme using a combination of online and face to face sessions, incorporating interactive and experiential learning methods. Each practitioner will receive a comprehensive training manual (47) that outlines session formats – including focus setting, targeted topic discussions, and progress evaluation (48) – alongside concrete examples to support delivery. These examples will guide practice in addressing common presenting problems identified among older women (49). To support skills development, we will provide one-to-one booster sessions, reflecting practitioner feedback and evidence that such sessions enhance the effectiveness of DA training (15, 49) by strengthening clinical skills (50). Group supervision will be provided throughout the delivery of A-PATH, using a hybrid model to accommodate online and in-person participation. Data from both sets of focus groups (Part 1 and Part 2) will be analysed using framework analysis (51) to identify key adaptations needed to ensure the intervention is appropriately tailored to the specific needs of older women.

Work Stream 2

Identifying suitable referral pathways

This Work Stream aims to improve access to psychological support for older women affected by DA by developing clear, setting-specific referral pathways across general practice, emergency departments, acute hospital wards, and community-based services (52). These settings are often the first point of contact for older women, where disclosures may occur in the context of experienced physical or mental health concerns. However, responses can be inconsistent, and older women's specific needs are frequently overlooked. Creating clear, setting-appropriate referral pathways is critical to ensuring timely, safe and supportive access to psychological interventions like A-PATH. This approach can help address existing service gaps and reduce missed opportunities for intervention. Informed by early stakeholder engagement, this will likely include the co-development of a referral protocol outlining clear referral criteria, practitioner guidance and structured signposting (53). To further inform this work, we will conduct individual interviews with survivors (n=10) and shadow helpline phone calls (n=10) in collaboration with our partner Hourglass. These will focus on identifying the structural and practical

challenges older women face when accessing support, and on developing strategies to help address and overcome them.

We will conduct two focus groups involving a diverse range of stakeholders, including GPs, nurses, psychiatrists, clinical psychologists, social workers, DA practitioners, third sector representatives, and survivors (n = 7- 8 participants per group; total n = 15), to explore: 1) the suitability of current referral pathways, with a focus on identifying barriers and facilitators that affect access to support for older women, and 2) the development of new referral pathways including practical guidance for use in specific settings. We will ask professionals and survivors how A-PATH can fit into existing services and how collaboration between primary care and community-based organisations can be strengthened (54). Prior to our feasibility trial we will examine referral routes into A-PATH to ensure they are accessible.

Data will be analysed thematically using both deductive approaches – based on pre-specified research questions – and inductive analysis to capture insights emerging from participants' accounts (51, 55). Emerging themes will be reviewed and validated by our LEAP experts. To support meaningful participation and relationship-building, we will allocate time early in the project to develop trust through community engagement strategies. This will include working with health ambassadors and community research champions who can act as trusted intermediaries, helping to foster relationships, and ensure the research is accessible, and inclusive (56, 57).

Work Stream 3

Feasibility and acceptability study of A-PATH

We will pilot A-PATH using an individually RCT design (58), in which eligible older women will be randomly assigned to receive either the adapted PATH intervention (plus DA advocacy) or usual care (DA advocacy only). We will assess the suitability of outcome measures, identify an appropriate primary outcome, and explore the acceptability of randomisation for older women affected by DA.

The primary aim is to assess whether a fully powered large-scale study is feasible and acceptable, by collecting information about the recruitment and trial retention rates that may affect the main trial. We will establish recruitment pathways, and identify facilitators and barriers to recruitment, including whether specific formats of the intervention (e.g. online or materials written in English) present a barrier to older women. Estimation of the (study) retention rate will enable us to assess whether a main trial is feasible, and to adjust for withdrawal appropriately. We will evaluate the feasibility of collecting outcome data, and explore participant burden, length of time and difficulty in completing measures and missing data. We will consider how best to balance and stratify our randomisation (e.g., by age, co-habitation status, and site), with our sites chosen to increase diversity and serve both rural and urban areas.

Target population and inclusion/exclusion criteria

We will recruit women aged 60 years or older who have experienced or continue to experience DA. Our study will focus specifically on intimate partner violence in later life, rather than broader forms of DA such as abuse by adult children or other family members. Our inclusion criteria will be as follows:

- women aged 60 years or older

- who have experienced domestic abuse, including physical, emotional, psychological, sexual, or financial abuse, either currently or in the past
- are able to provide informed consent to participate in the study
- have sufficient cognitive capacity to engage in the intervention and complete study assessments.

Although we will try to be as inclusive as possible, our PPI pre-grant advisory panel have advised us that we will need to exclude women with a psychotic illness, those living with a severe drug or alcohol problem, or those living with dementia. Individuals currently engaged in other structured psychotherapeutic treatments will also be excluded. We will only recruit older women, given evidence that they are more likely to experience repeated and severe forms of abuse compared to older male victims (59). We will purposively recruit older survivors from linguistically and culturally diverse backgrounds (e.g., South and West London), with participating services drawn from varied geographical areas across England and Wales. These sites reflect differing levels of prior engagement with older survivors, enhancing the diversity of perspectives and increasing the generalisability of our findings. For the purposes of adapting PATH, we define 'older women' as those aged 60 and above—a threshold that reflects the distinct social, psychological, and health-related experiences associated with later life, and aligns with best practice in DA advocacy for older survivors (18, 19).

Sample size calculation

As this is a feasibility study, the primary aim is not to detect a statistically significant effect but to estimate key parameters necessary for designing a future large scale trial, including acceptability of the intervention, and sensitivity to change of outcome measures (60, 61). Therefore, a formal power calculation is not appropriate at this stage. We will randomly allocate thirty-eight survivors to DA advocacy (usual care), and thirty-eight to DA advocacy plus A-PATH (n = 76). Our proposed sample size is based on recommended guidance for feasibility trials (58, 61), allowing sufficient precision to estimate feasibility outcomes, and gather preliminary data on outcome measures (e.g., mental health, wellbeing, service use). Findings from this feasibility trial will inform robust sample size calculations for a future large-scale RCT by providing estimates of effect size variability and other key design trial parameters.

The sample size of 76 that we have chosen allows us to estimate a retention rate of 75% at 8 months to within a 95% confidence interval of ±10%. This sample size will also enable us to estimate the standard deviation of our candidate primary outcome (Clinical Outcomes in Routine Evaluation–Outcome Measure (CORE-OM); psychological distress; (62, 63)) in our target population with sufficient precision (64). Based on existing data, we expect the standard deviation of CORE-OM to be approximately 5 (63), which we will estimate within a 95% confidence interval of ±0.9. The standard deviation estimate will inform the sample size calculation for our future larger trial. These sample size calculations were performed in Stata v18. These retention rates align with those observed in previous trials identified in our review, demonstrating their feasibility and achievability (29, 65). Outcome measures will be administered at baseline (prior to randomisation), at 4, 8 (primary outcome

timepoint), and 12 months post-randomisation to assess changes over time and inform follow-up feasibility.

Randomisation will be conducted by an unblinded member of the research team using a secure online randomisation system (64). In cases where a practitioner or researcher identifies a significant risk of self-harm, suicide, or harm from others, unblinding may be required to ensure appropriate and timely support for the participant's safety and well-being. To ensure the safety of both participants and the research team, we will develop and implement comprehensive risk management protocols, which we will adapt from our previous and ongoing studies (e.g. REPROVIDE (66)). Two distinct protocols will be established: one to guide responses to participant distress during or after study activities, and another to address safeguarding concerns related to ongoing abuse or risk of harm. These protocols will be informed by best practice guidelines and developed in consultation with our clinical (CH) and safeguarding (AW) leads to ensure a trauma-informed and ethically robust approach throughout the study.

Primary outcomes

Feasibility of progressing to a multi-centre RCT will be assessed primarily through recruitment and retention of participants, as well as acceptability and absence of any major concerns such as multiple serious adverse events. Satisfaction on these key points will indicate the practicality of scaling up the intervention.

Primary feasibility outcomes:

We will collect data on: a) the number of referrals and proportion of women recruited and randomised to the study out of those referred, b) the proportion of those retained at 4, 8, and 12 months, and c) the extent to which participants engage with the adapted PATH intervention as intended. Quantitative metrics which will include the percentage retained at primary endpoint (8 months) will be analysed alongside qualitative insights gathered through interviews with participants (both older women and practitioners) who withdraw or are lost to follow-up. This mixed-methods approach will enable us to assess the feasibility of achieving adequate retention and inform appropriate strategies to optimise it in a future full-scale RCT. Our stop/go progression criteria for retention (60) (baseline to primary outcome completion at 8-month follow-up out of number randomised) will be as follows:

a) trial retention rate approaching 80% will be considered supportive of feasibility for progression to a full trial
b) trial retention between 65% - < 80% indicative that a full trial is feasible, provided reasons for poor retention are identified and addressed, and
c) trial retention rates below 65% as indicative that our feasibility study is unlikely to progress to a full trial.

Intervention adherence will be assessed through session attendance, completion of core intervention components, and insights from qualitative follow-up interviews. We will explore reasons for non-attendance, with particular attention to barriers experienced by older women, to inform adaptations to the PATH intervention that enhance accessibility, engagement, and relevance for this population. As part of tailoring the PATH intervention to meet the specific needs of older women, we will adapt an existing fidelity framework to ensure core therapeutic components are delivered consistently while allowing appropriate flexibility for individual needs and

context (30, 31). This adapted framework will define essential elements of the intervention, identify adaptations needed (e.g. delivery mode, pacing, language), and include mechanisms for monitoring fidelity through practitioner training, supervision, and session checklists. This approach will help maintain intervention integrity while supporting relevance and responsiveness to older women's diverse experiences and preferences (35).

Secondary outcomes

As part of our feasibility RCT we will try to identify the most appropriate primary outcome measure for our large-scale multi-centre evaluation trial. We will estimate potential effectiveness on a range of candidate primary outcome measures, and their standard deviations (SDs). These will include:

a) psychological distress - Clinical Outcomes in Routine Evaluation–Outcome Measure (CORE-OM) (62); The 34-item CORE-OM assesses global psychological distress and is validated for sensitivity to change, test–retest reliability and UK normative comparison. Originally developed to evaluate the effectiveness of psychological therapies from several disciplines (67), it will be used in this study both as a continuous outcome (mean score) and a binary variable, using the clinical cut-off score of ≥ 9.9 (62).

b) depressive symptoms - Patient Health Questionnaire (PHQ) (68); The nine-item PHQ assesses depressive symptoms and demonstrates strong validity, reliability and sensitivity to change in older adults (69). In this study, it will be used as both a continuous measure (mean score) and a binary outcome based on the established clinical cut-off score of ≥10, indicative of probable major depression (69).

c) post-traumatic stress symptoms – Posttraumatic Stress Disorder Symptoms Scale (70); this 17-item questionnaire measures post-traumatic stress symptoms with a score ≥ 17 consistent with a clinical diagnosis of PTSD and has established validity in older samples (71), and

d) health-related quality of life - EQ-5D-5L (72); the EQ-5D-5L is a widely validated, standardised instrument that captures core dimensions of health, making it suitable for use across a range of populations and health conditions.

A ‘shortlist’ of potential primary outcome measures will be developed working closely with our LEAP group. We will explore participants’ and practitioners’ views of their suitability in the qualitative follow-up interviews. Participants will also be asked to record key demographic data at baseline and complete a brief feedback form when they finish the A-PATH sessions.

We consulted with lived experience experts to help us determine our primary outcome timepoint. Their feedback emphasised the importance of allowing enough time to observe meaningful and sustainable change. This input supported the selection of the 8-month timepoint as our primary outcome timepoint, ensuring participants have ample opportunity to complete A-PATH and to apply its content in their daily lives. This was considered especially important given the complexity of DA in later life. An 8-month follow-up allows us to capture longer-term indicators of impact and is supported by other DA intervention RCTs (15, 25, 30), and NIHR feasibility study guidance (41). We will also collect data at 12 months to assess sustained engagement and outcomes, with this timepoint serving as the primary outcome measure in the full-scale trial, consistent with our PROVIDE trial (30, 31).

Health-economic analysis

We will explore feasibility of collecting economic data, and establish the optimum way of evaluating cost-effectiveness in a multi-centre evaluation trial. Our health economic evaluation will consider the perspectives of health and social care, public sector and society, using Quality-Adjusted Life Years derived from the EQ-5D (72) and a modified Client Service Receipt Inventory (73). We will tailor our resource use questionnaire, in consultation with our PPI group and refine it through early testing. We will explore the completeness, acceptability, and relevance of these measures, including the feasibility of collecting data from both NHS and wider service perspectives.

Process evaluation

We will use realist methodology to identify what works, for whom and in what circumstances (74), utilising the Medical Research Council (MRC) process evaluation framework (75). A key focus of the process evaluation will be to explore the acceptability of the A-PATH intervention, including how it is experienced by participants and practitioners within real-world settings. We will conduct qualitative process interviews with up to 15 survivors who participated in the A-PATH intervention, capturing a range of engagement levels, alongside interviews with up to 10 survivors from the usual care group to explore comparative experiences. Where possible, we will also conduct short informal 'exit' interviews (participants who drop out of the intervention or study), to better understand reasons for disengagement. We will interview DA practitioners (n = 6) to gather insights into what elements of the intervention are working well, and what needs to improve, and to inform our understanding of A-PATH's mechanisms of change (31).

The process evaluation work will enable us to identify candidate factors that may require adaptation within the PATH protocol, thereby informing refinements aimed at improving retention among older women in a future large trial. We will explore DA practitioners' perspectives on factors that facilitate or hinder engagement among diverse groups of older women and how obstacles can be overcome (76). We will use this data to inform the refinement of our logic model to be fully tested in a future large-scale RCT (43). A comprehensive flow chart detailing all stages of the research is presented in page 20. This final Work Stream will synthesise the study's key outputs, including the adapted intervention, an updated logic model, a co-produced referral pathway, and the delivery of a feasibility pilot RCT.

Health technologies being assessed

A-PATH Intervention

Women randomised to A-PATH will be offered up to eight weekly or fortnightly sessions, followed by two booster sessions, 1 month and then 3 months post-intervention. The schedule will be flexible and revised according to participant preferences. A-PATH is designed to support women in recognising, and exploring psychological consequences related to experiences of DA, while developing coping strategies to manage conflict and other stressors (30, 31). In collaboration with survivors and practitioners we have identified several adapted topics relevant to older women, including "What I Want to Happen" and "Planning for the Future" which will be incorporated into the intervention content. The therapeutic sessions will be

delivered alongside standard DA advocacy support. When adapting PATH, we will draw on the '*psychosocial readiness mode*l' of change (77), which highlights awareness, perceived support and perceived power as key factors influencing behaviour change in women affected by DA. As part of this work, we will produce a revised logic model with specific behaviour change components for this group.

Recognising the diverse barriers faced by older women including digital exclusion, mobility limitations, frailty, and social isolation—the intervention will employ a flexible delivery model that accommodates individual needs and preferences. Participants will be offered flexible access to the intervention through multiple formats—face-to-face sessions in accessible community or healthcare settings, telephone-based therapy, or online video calls—ensuring it is both feasible and tailored to individual preferences. Insights from our lived experiences study indicated that this flexibility ensures that women without internet access or digital skills (e.g. those aged over 80; (78)), can still engage fully, while those comfortable with technology have the option of remote support. All DA practitioners will receive regular supervision by RAD (25), throughout the study period.

Usual care

Women in the control group will receive usual care in the form of DA advocacy support but will not participate in A-PATH sessions. The advocacy support will include a package of legal, housing, financial, and safety advice, provided by our partner Hourglass, a leading national charity and experts in DA of older women (8). This support will be provided by trained DA practitioners to ensure women receive accurate, timely, and trauma-informed guidance. The package is designed to empower survivors with critical knowledge about their rights, options for financial independence, and personalised safety strategies, and support women in making informed decisions about their future. Advocates in this group will not receive specialist training in psychological methods.

What might the A-PATH intervention look like*?

A-PATH is designed to be **flexible** and **client-led**. In the first session, the practitioner and client explore the psychological impact of abuse by focusing on **validating lived experiences.** Subsequent sessions are collaboratively selected by the client and practitioner, ensuring A-PATH remains responsive and person-centred. Core themes include **strengthening self-esteem**, **promoting self-care**, **reducing trauma and psychological distress**, and **building emotional resilience** to support recovery and wellbeing. The final session focuses on reviewing progress, reinforcing key strategies, and **planning for the future.**

** Informed by the original PATH trial, our qualitative research, lived experiences, and wider evidence on tailoring DA interventions for older women (15, 25)*

Knowledge mobilisation, dissemination and impact

Knowledge mobilisation

Despite progress in DA research and policy, older women survivors continue to be marginalised, and experience social, economic, emotional, and institutional exclusion (19). Our aim is to translate research into meaningful action by:

a) building an interactive A-PATH website, with survivors and practitioners, to share findings in accessible, engaging formats and actively invite feedback from a broad public and professional audience;
b) collaboratively developing a set of evidence-informed referral pathways for use across the NHS, local authorities, and the third sector;
c) facilitating co-designed workshops and roundtable discussions with survivors, frontline professionals, and community organisations to reflect on our findings and collaboratively shape the design of a future large-scale trial.

We will follow guidance on community-based knowledge mobilisation in DA research (79, 80). Central to our approach is the development of equitable partnerships with survivors, practitioners, and third-sector organisations, recognising their essential roles in shaping, delivering, and sustaining meaningful change.

Dissemination

Working with key stakeholders, and lived experience experts we will co-develop a dissemination plan to identify who we need to engage, how to reach them and ensure outputs are accessible and inclusive. We will draw on our National Peer Research Network for Preventing Domestic Abuse in Older People, hosted by UCL, which brings together survivors, researchers, practitioners, and community organisations across the UK. The network will help us reach a broad and diverse audience including health and social care professionals and third-sector organisations. Our dissemination activities will be facilitated with the support of our LEAP experts and third sector partners. We will publish our trial findings in peer-reviewed academic journals and present these in national DA conferences. We will attend local community events attended by commissioners of DA services to present our results and host a public knowledge exchange event to share our findings and our 'good practice' referral pathway.

Impact

This feasibility trial will provide critical preparatory work for addressing a major gap in supporting older women affected by DA. We anticipate direct impact on improving awareness of DA affecting older women, and guidance on suitable referral pathways including videos describing our key findings. By adapting the evidence-based PATH intervention to meet the specific needs of older women, this study will improve its future accessibility by tailoring its content to the unique experiences, health profiles and barriers faced by older women. This study will also strengthen the current evidence base by generating essential data on recruitment, retention, and outcome measurement in this population. Developing an intervention that is shaped by lived experience and is responsive to the psychological, social and practical needs of older women allows us to amplify the voices of older survivors, ensuring we lay the foundations for scalable change in DA support services and practice across health and third-sector settings. If A-PATH is feasible and acceptable, it will lead to a larger HTA trial evaluating the first evidence-based trauma-informed and age-responsive psychological intervention for older women affected by DA.

Project management

Timeline, and milestones of the project

The project will be delivered over 36 months. The first 9 months will focus on adapting the PATH intervention and co-developing setting-specific referral pathways. This will be followed by a 23-month feasibility study, including an 8-month recruitment phase and a 12-month outcome assessment period. In the final 6 months we will complete data analysis, interpretation, and dissemination through reports, academic publications, and stakeholder engagement.

Work Stream 0 (Months 3–0): We will obtain all necessary NHS Ethics and Health Research Authority (HRA) approvals between October and December 2025—three months before the planned study start date—to ensure full regulatory compliance and allow adequate time for site setup and preparatory activities.

Work Stream 1 (Months 1–4): We will conduct two participatory focus groups and a roundtable discussion with key stakeholders to inform the adaptation of the PATH intervention. During this phase, we will also revise the original training package for DA practitioners to ensure it is relevant for supporting older women. During this WS we will field-test the adapted PATH intervention to gather preliminary data on barriers and facilitators in real-world settings.

Work Stream 2 (Months 5–9): We will conduct individual interviews with survivors and focus groups with professionals to explore and optimise setting-specific referral protocols across services such as general practice, emergency care, acute wards, and community settings.

Work Stream 3 (Months 10–36): A 23-month feasibility study will be undertaken (Months 10–23), including a 8-month recruitment phase (Months 10–17), supported by our established National Network, third sector partners and participating NHS Trusts. Training for DA practitioners will take place before recruitment begins to ensure preparedness across sites. Quantitative and qualitative data will be analysed and disseminated through academic publications (Months 31-36), conference presentations and community stakeholder workshops.

Recruitment centres and partners

Building connections to support recruitment. Recruitment of older women affected by DA requires building trusted connections with services and communities that support this population. We will engage with NHS settings (e.g. general practice, and mental health services), third-sector organisations and community groups working with older adults. These partnerships will help us identify potential participants sensitively and ethically, while ensuring outreach is inclusive of women from diverse backgrounds, drawing upon established IRIS (Identification and Referral to Improve Safety) sites (34) and our existing networks. Our feasibility trial will be delivered across **four principal recruitment centres**—London, Bristol, Swansea, and North Devon—**each supported by two affiliated local recruitment sites (NHS and community-based),** all with established research infrastructure.

- **London (University College London; 4 sites; projected recruitment 32 participants)**: UCL brings extensive experience in leading complex mental health and DA trials. It has well-established NHS and community sector partnerships to support recruitment and delivery, and led our lived experiences study. This centre will recruit from the North West London NHS Foundation Trust, Camden and Islington NHS Trust (and partner Trusts through Noclor), and local centres (Age UK, North London Women's Aid and SafeLives), contributing a total of 4 individual sites.

- **Bristol (University of Bristol; 2 sites; projected recruitment 16 participants)**: University of Bristol is a national and international leader in DA research. The team at Bristol has longstanding collaborations with local services and a proven track record of delivering intervention studies with women affected by DA. The Bristol team will recruit from NHS Bristol, North Somerset and South Gloucestershire Trust, and local DA services that participate in ongoing trials (Bristol Next Link).
- **Swansea (Dewis Choice; 2 sites; projected recruitment 14 participants)**: Dewis Choice is the only UK initiative specifically designed for older survivors of DA, with expertise in age-informed, trauma-sensitive practice. This site will work closely with the Centre for Age, Gender and Social Justice, Solace Women's Aid and the Swansea Bay University Health Board.
- **North Devon (North Devon Against Domestic Abuse – NDADA; 2 sites; projected recruitment 14 participants)**: As a trusted local service with embedded links to health and social care providers, NDADA offers crucial insight into rural service delivery for older women affected by DA in underserved areas. This site will receive referrals directly from NDADA and the Royal Devon University Healthcare NHS Foundation Trust.

Our four recruitment centres provide geographic and demographic diversity, supported by strong community and clinical networks across ten active recruitment sites. Based on empirical benchmarks from NIHR feasibility studies (81, 82), we anticipate a recruitment rate of one participant per site per month. Across ten sites over 8 months—the recommended window for engaging underserved groups—this provides a potential total of 80 participants, allowing a buffer above our target of 76 to mitigate potential recruitment challenges. This estimate reflects realistic feasibility assumptions (61) and is enabled by our plans to include national recruitment pathways e.g. via Hourglass helplines and an online delivery option.

Third sector partners

To ensure inclusive recruitment, we will collaborate with key third sector organisations that have established expertise in supporting older women affected by DA. These include **Hourglass**, the only UK-wide charity dedicated to ending abuse and neglect of older people; **Women's Aid**, a national federation supporting women experiencing DA; **SafeLives**, a leading organisation with cross-sector frontline expertise; **Age UK**, which delivers safeguarding and support services for older adults; and **Dewis Choice**, a pioneering Welsh initiative co-produced with older survivors and led by one of our co-applicants. We will also recruit through **North Devon Against Domestic Abuse (NDADA**), a specialist provider of refuge and community-based support for women in rural and underserved areas. These trusted partners will support participant identification, DA practitioner recruitment, and help develop trauma-informed, age-sensitive engagement strategies. All materials will be co-designed with our LEAP group to ensure they are accessible, respectful, and reflective of older women's experiences. The longstanding credibility and community presence of our partners will be central to improving reach, trust, and engagement with underserved populations.

NHS Trust partners

Recruitment through NHS Trusts will be a key component of our feasibility trial. We will work closely with general practice teams including mental health services, community mental health older adult teams, safeguarding leads, and health-based independent domestic violence advisors (IDVAs). These professionals are well placed to identify older women who may be experiencing DA but are not currently accessing specialist support. Site-specific research champions from our National Network will promote the study among frontline staff.

Recruitment of DA Practitioners

We have already identified 1 practitioner (DA champion) in each of our sites. The remaining practitioners will be identified through established DA support networks, our partner Hourglass, specialist services, and community organisations and will be led by our safeguarding lead (AW) and our DA champions. We will adopt a proactive, evidence-based recruitment strategy aligned with our previous trial (30), and the landmark IRIS study (54), where each study site has a designated advocate coordinating referrals. Our community-led approach—centred on collaboration with survivors, frontline professionals, and voluntary sector leaders—places us in a strong position to deliver this research effectively. Community outreach will be a key component of our engagement strategy (83), supported by our National peer research network preventing domestic abuse in older people initiative.

Study management, and expertise in the study team

Our multidisciplinary team brings together expertise from psychology, social science, methodology, statistics, general practice, nursing, DA advocacy and lived experience. Collectively, we have a strong track record in the development and evaluation of complex, community-based interventions for older adults and women affected by DA. Our team includes Dr Vasiliki Orgeta, a specialist in psychological interventions in later life (29), who co-led the recent Lancet Commission on Trauma and Ageing, which highlighted the underrepresentation of older women affected by DA in both research and policy (84). Dr Helen Cramer is a nationally recognised leader in perpetrator-focused interventions (57, 66). They will receive institutional support from both University College London and the University of Bristol—centres of excellence in older adults' mental health and DA research.

Dr Roxane Agnew-Davies is a clinical psychologist with over 30 years of experience in specialist mental health services for women affected by DA. She led the development of PATH implemented in various sites across England and Wales and has published widely on DA. Dr Alison Gregory is a nationally recognised expert on the secondary trauma experienced by informal supporters of DA survivors. She has led multiple impactful research projects with strong partnerships with Women's Aid, local authorities, and public policy stakeholders to translate research into practice.

Dr Victoria Vickerstaff brings expertise in statistical methodology and applied health research, with a robust track record of leading the design and analysis of several multidisciplinary NIHR-funded trials in older people and palliative care settings. Amanda Warburton-Wynn is the Domestic Abuse Health Strategic Lead for Cambridgeshire County Council, working with health agencies across the county to improve the recognition and response to domestic abuse. She developed the Older People's DASH risk assessment tool for DA that has been used in Cambridgeshire

since 2021 and is currently being piloted in Norfolk and Wales. She is also an independent researcher, specialising in improving support for older people and people with disabilities who are subjected to DA and sexual violence.

Marion Goodchild brings over 25 years of experience in nursing with a strong track record of championing patient-centred approaches in health and social care research, such as enhanced training for cancer care teams in identifying DA. She has contributed to multiple research programmes as a public advisor, helping to improve participant engagement and inclusive practice. Dr Claire Hawcroft is a practising GP and clinical academic with expertise in the design and evaluation of trauma-informed interventions for women affected by DA across primary care and community settings. Professor Sarah Wydall is a leading expert on DA in later life. As Principal Investigator of the Dewis Choice project, she has pioneered multi-agency, participatory responses for older survivors, including those with dementia, and developed innovative tools and frameworks now used nationally.

Data management, reporting and analysis

Questionnaire and clinical data will be collected and stored securely in REDCap, with access restricted to authorised research team members. Administrative data—such as participant names, contact details, and follow-up information—will be pseudonymised and stored in a separate, password-protected database (e.g. Microsoft Access), in accordance with data protection regulations. This separation of identifiable and research data ensures confidentiality, minimises risk of data breaches, and complies with Good Clinical Practice (GCP). All personal data, and data handling, processing, storage, and destruction will comply with the Data Protection Act 2018 and GDPR regulations. Data quality assurance procedures will be in place to verify the accuracy of information entered from any hard-copy questionnaires. Recruitment and retention data will be summarised using a CONSORT flow diagram. A statistical and health economic analysis plan for the feasibility trial will be developed by the trial statistician (VV) in collaboration with the Trial Management Group.

Study oversight and safety protocol

Oversight will be provided by our LEAP panel, which will meet quarterly to inform and contribute to all Work Streams. The Programme Management Group (PMG) will meet monthly throughout the study to oversee delivery, with a particular focus on monitoring recruitment against projected targets. Meetings will include regular review of site-level performance, identification of emerging challenges, and implementation of corrective actions where necessary to ensure timely and effective recruitment. Additional ad hoc meetings will be convened should any significant issues arise.

The quality and integrity of trial data will be monitored by an independent Data Monitoring and Ethics Committee (DMEC) and a Trial Steering Committee (TSC). A comprehensive safety protocol, aligned with WHO guidelines (85), will be used to record and monitor adverse events. The TSC will review any reported harms associated with the A-PATH intervention. All serious adverse events will be reported immediately to the sponsor, TSC, and the NHS Research Ethics Committee (REC) in accordance with the UK Policy Framework for Health and Social Care Research (2020). Participant safety will be overseen by our designated Safeguarding Lead

(AW), who will conduct monthly safety reviews and report findings to both the PMG and LEAP.

If participants (older women or practitioners) become distressed during any sessions or assessments, the researcher/DA practitioner will respond in line with established safeguarding and distress protocols. Immediate emotional support will be offered, and the session paused or discontinued if needed. With the participant's consent, a referral will be made to appropriate support services, including local DA specialists and mental health providers. All research staff will be trained in trauma-informed approaches and will follow a clear distress and a separate safeguarding protocol approved by the ethics committee. Our study is informed by best practice guidelines from UK third sector organisations, including SafeLives, Women's Aid, and AVA (Against Violence and Abuse), emphasising the importance of conducting research that is trauma-informed, survivor-centred, and grounded in principles of safety and confidentiality. Researchers will be trained to recognise distress, and prioritise participant autonomy and well-being. All procedures will align with the ethical standards outlined by the UK Research Integrity Office (UKRIO) and the HRA guidance for research involving vulnerable adults, with input from our lead specialist DA advisor (AW) during our protocol development.

Equality, diversity, and inclusion

We have considerable expertise in recruiting participants from diverse communities. We will engage with seldom heard populations (e.g., older women from Black, Asian and minoritised ethnic communities, those that identify as LGBTQIA+), which face unique barriers in coping with abuse (76, 86). To encourage recruitment from non-English speaking communities, we will implement inclusive and culturally sensitive engagement strategies throughout the study. We will work with established leaders (e.g., Hourglass, Women's Aid), to ensure wording, images, and recruitment methods are as inclusive as possible. Recruitment materials (i.e., participant information sheets, posters, briefings to teams) will be reviewed for cultural appropriateness. We will work in partnership with community organisations that have established trust within ethnic minority communities to raise awareness of the study. We will work closely with local services and faith groups that support minoritised communities, to increase our insight of how best to serve these communities. We will collect data on individuals who were potentially eligible but excluded due to limited English proficiency, to inform inclusive recruitment strategies for a full-scale RCT, ultimately improving the generalisability of the adapted PATH intervention.

Intellectual property and commercialisation

All novel materials developed during this project will be identified and documented as potential intellectual property (IP). We will work closely with UCL's IP team to develop our IP strategy, and ensure that any outputs maximise public benefit while maintaining accessibility for organisations who support older women affected by DA. The overarching aim is to ensure that A-PATH delivers lasting value — improving outcomes for older survivors, and equips third-sector organisations with the tools to support their recovery and well-being**.** We will explore opportunities to expand the programme's reach through partnerships with third-sector organisations, licensing arrangements, or social enterprise models.

Logic model of A-PATH - Adapting Psychological Advocacy Towards Healing for older women affected by domestic abuse

Population

Population:
Women aged 60 years or more

Physical health risks
- Increased risk of frailty
- Increased incidence of chronic conditions

Psychological harm
- Depression, anxiety and trauma symptoms
- Shame & self-blame

Social risks
- Isolation
- Stigma
- Lack of support

Financial harm
- Economic abuse

Systemic risks
- Ageism in services
- Limited access to support

Theory, Assumptions & Implications

Empowerment Theory
- Building autonomy
- Enhancing self-efficacy
- Challenging systemic barriers

Assumptions & implications
- Gaining control
- Building confidence
- Promoting healthy relationships
- Empowerment through access to knowledge
- Autonomy and voice

Approach
- Focusing on strengths
- Avoiding paternalism
- Clear & accessible information
- Challenging ageism, stigma and exclusion

Intervention

A-PATH intervention
- Uses the psychosocial readiness model & CBT principles
- Strategies to cope with distress
- Enhances self-esteem
- Strengthens resilience
- Promotes self-care

Mechanisms of change
- Awareness
- Perceived support
- Perceived power

Control group
- Receives usual DA advocacy

DA advocacy
- Practical, legal, financial & safety advice
- Delivered by third sector and local DA services

Short Term outcomes

Psychological well-being:
- Improved self-esteem
- Reduced psychological distress

Sense of control
- Increased confidence
- Increased self-efficacy
- Greater control over routines & choices

Social connection
- Reduced social isolation
- Improved trust in others
- Rebuilding relationships

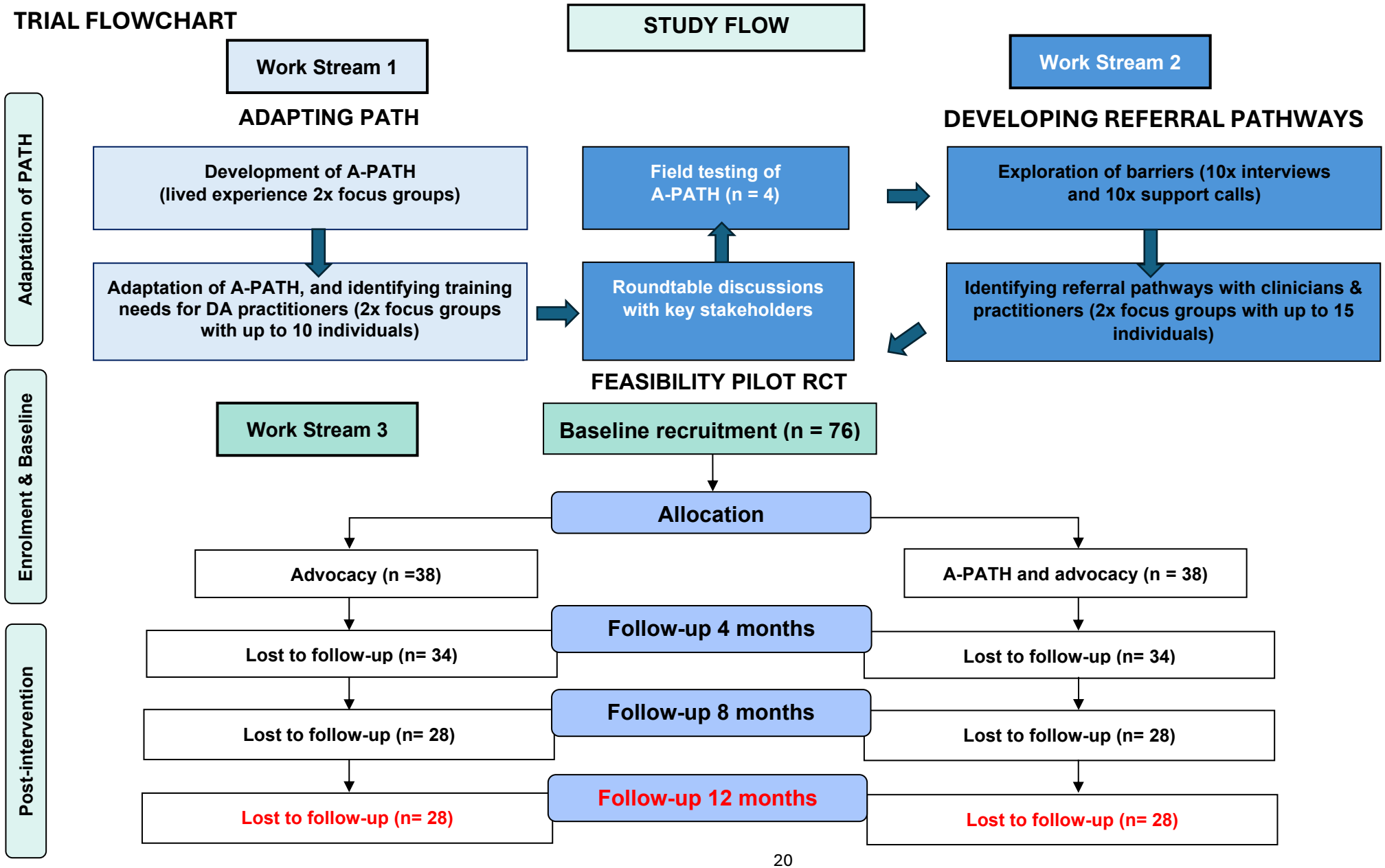
Long Term Outcomes

Psychological well-being:
- Reduce depression
- Reduce trauma symptoms
- Living with dignity
- Thriving beyond safety
- Improved well-being
- Strengthening resilience

Safety
- Awareness of risk
- Reduction in exposure to harm

Services engagement
- Increased awareness of services
- Improving ability to navigate services

Structural change
- Inclusive care systems
- Tackling ageism and stigma


TRIAL FLOWCHART
STUDY FLOW
Work Stream 1
Work Stream 2
ADAPTING PATH
DEVELOPING REFERRAL PATHWAYS
Adaptation of PATH
Development of A-PATH
(lived experience 2x focus groups)
Field testing of
A-PATH (n = 4)
Exploration of barriers (10x interviews
and 10x support calls)
Adaptation of A-PATH, and identifying training
needs for DA practitioners (2x focus groups
with up to 10 individuals)
Roundtable discussions
with key stakeholders
Identifying referral pathways with clinicians &
practitioners (2x focus groups with up to 15
individuals)
FEASIBILITY PILOT RCT
Enrolment & Baseline
Work Stream 3
Baseline recruitment (n = 76)
Allocation
Advocacy (n =38)
A-PATH and advocacy (n = 38)
Post-intervention
Follow-up 4 months
Lost to follow-up (n= 34)
Lost to follow-up (n= 34)
Follow-up 8 months
Lost to follow-up (n= 28)
Lost to follow-up (n= 28)
Follow-up 12 months
Lost to follow-up (n= 28)
Lost to follow-up (n= 28)

A-PATH: Adapting Psychological Advocacy Towards Healing for older women affected by domestic abuse

Background and rationale

Data published by Age UK in 2023 indicate that more than 375,000 older women aged 60 to 74 experience domestic abuse (DA) in England and Wales (1). The 2025 Femicide Census has revealed that one in eight women killed by men in the past 15 years was aged over 70 (2). DA can lead to serious physical health injuries for older women (3), often associated with prolonged recovery periods and an increased risk of mortality (4, 5). DA has profound emotional and psychological impacts, including increased severity of anxiety and depression (4), which significantly diminish older women's well-being and quality of life (6). Despite a growing number of older women experiencing DA, reports published by our partner Hourglass (7, 8) highlight persistent gaps in support and mental health provision for older women (9).

DA can expose older women to a range of harmful behaviours that are frequently underreported and poorly recognised within services (10, 11). These may include physical and emotional abuse – such as sustained verbal belittlement and psychological manipulation – as well as sexual abuse involving coercive or non-consensual acts, often occurring within long-term relationships (12, 13). Financial abuse is also common, typically involving the misuse of pensions, or the restriction of access to personal funds (14). Controlling behaviours – such as limiting access to healthcare or isolating individuals from social networks – further undermine older women's autonomy and well-being (14). These forms of abuse are often exacerbated by generational norms, such as the expectation that older women should endure hardship, and by systemic barriers, such as digital exclusion and insufficient training among professionals, which together limit access to timely support and care (15, 16).

The unique barriers faced by older women affected by DA are frequently overlooked in mainstream service provision and academic research (17). Abuse may span decades, exposing women to cumulative trauma, and become embedded in long-term relationships, making it harder to recognise or disclose (18). Many women may not recognise controlling behaviour or financial coercion by a long-term partner as abuse, particularly when such patterns have been normalised over time (19). For some older women, disclosing abuse may carry a significant risk of losing their home, financial security, or contact with grandchildren (20). Living with a chronic condition or frailty, increase dependence on the perpetrator, thereby reducing opportunities for older women to seek help (4).

Health and social care professionals sometimes miss signs of abuse, often attributing injuries or distress to age-related conditions or frailty rather than recognising them as intentional harm (9, 21). Our qualitative study has shown that digital exclusion—caused by limited access to technology or digital literacy (22)—prevents many older women from accessing support, or engaging safely with remote interventions (18). These factors not only increase older women's vulnerability to sustained and repeated abuse, but also highlight the need for targeted support (23), so that older women can get the help they need (18).

Psychological interventions are a vital component of the care pathway for women experiencing DA (24, 25), addressing the significant mental health impacts such as

trauma, and depression (19, 26). While these interventions are typically accessed once immediate safety needs have been met (15, 27), there is growing evidence supporting their value earlier in the help-seeking journey, including for women who remain in abusive relationships but are beginning to engage with services (15). In these contexts, psychological support can help build emotional resilience, strengthen coping strategies, and support informed decision-making around safety and self-care (25).

Psychological interventions that are not tailored to meet the specific needs of older women are less likely to be useful to female survivors and may even be harmful (28). Such harm can include retraumatisation, feelings of alienation or invalidation, and reduced engagement with services perpetuating health inequalities faced by older women (20, 27). Our review reporting on the clinical effectiveness of psychological treatments for older people exposed to trauma, found no interventions for older women who have experienced DA (29). This project aims to address this critical service gap by adapting an existing evidence-based psychological intervention - Psychological Advocacy Towards Healing (PATH) - originally developed for women of childbearing age surviving abuse (30, 31), to meet the unique needs of older women.

Developed by Agnew-Davies and colleagues (32), PATH integrates cognitive behavioural therapy (CBT), experiential and feminist theoretical frameworks to equip women with practical skills for managing the psychological distress associated with abuse (31). Although the PATH intervention has demonstrated effectiveness in reducing psychological distress and depressive symptoms in women aged 18 to 67 (30), its suitability for older women remains unclear, particularly in addressing their distinct experiences and the specific barriers they face when seeking help (20). Our research has highlighted a range of unique and intersecting factors that make it harder for older women to access support and cope with abuse (9, 11). These include digital exclusion, frailty and social isolation (33), which are often exacerbated by ageism within health and social care services, and intensified by prolonged coercive control that restricts social networks (4). As part of this study, we will also seek to examine the need for and, as necessary, develop specific referral pathways for older women who experience DA. Improving access is an essential pre-requisite to more older women getting psychological support and more equitable access and service provision (25).

Although PATH promotes skills to enhance women's self-esteem, assertiveness, and resilience – core components of long-term recovery and empowerment (31), it lacks adaptation for age-related factors such as living with frailty and the complexities of lifelong abuse or late-onset abuse (13, 19). It does not address for example, barriers commonly faced by older women, such as digital exclusion, generational stigma around help-seeking, and dependence on abusers for care or housing (18, 19, 22). As a result, the standard PATH intervention may not be accessible, acceptable, or effective for this population without modification (34, 35). Adapting PATH to reflect the specific needs and circumstances of older women could significantly enhance its reach and relevance, extending the benefits of this evidence-based intervention to a significantly underserved group (31).

In focus group discussions with older women survivors, participants emphasised the value of accessing psychological therapies that can validate their experiences, and reported that they sometimes receive practical and emotional support through local statutory DA practitioners. Currently however, DA practitioners are not trained to deliver specialist psychological support, limiting the scope of care available to older women. In this project our aim is to train DA practitioners to deliver the adapted PATH intervention (A-PATH), enabling them to provide psychologically informed support alongside advocacy as usual (26).

Aims and objectives

Our overarching aim is to improve the mental health of older female survivors of DA, and support their well-being and recovery. To achieve this, we will: i) adapt the core components of the existing PATH intervention to ensure they are tailored to the needs and preferences of older women, (ii) strengthen and tailor referral pathways to enhance accessibility with support services, and, iii) determine the feasibility and acceptability of the adapted PATH intervention and trial design.

The objectives are to:

1. Adapt the PATH intervention specifically for older female survivors, including the development of appropriate referral pathways;
2. Identify the main variables that may influence the design of a full-scale trial, including factors affecting the evaluation of the cost effectiveness of this intervention;
3. Determine the acceptability and feasibility of the adapted PATH and trial design from the perspectives of clients and advocates and;
4. Explore the experiences and perceptions of those receiving and delivering the adapted intervention and its context.

Why is this research important in terms of improving the health and wellbeing of the public and health care services?

Older women affected by DA often access support through NHS services such as general practice (GP), emergency departments and hospital wards, where abuse may be disclosed or suspected during consultations related to physical or mental health concerns (36). The impact of DA in later life is often far-reaching, long-lasting and cumulative—exacerbating existing health conditions and contributing to the onset or worsening of mental health disorders (3, 6). Prolonged exposure to abuse can increase vulnerability to chronic health conditions, such as cardiovascular disease, musculoskeletal pain (37, 38) and elevates risk of frailty (39). Timely and tailored psychological interventions delivered by DA advocates have the potential not only to alleviate immediate psychological distress but also to reduce long-term health risks and subsequent service use among older women (40).

What is the evidence that the intervention is ready for HTA evaluation?

PATH is a manualised, trauma-informed intervention grounded in cognitive-behavioural principles, which is effective in reducing psychological distress and depression for women experiencing DA (25). This existing evidence base highlights the intervention's readiness for further adaptation and targeted evaluation, particularly for underserved groups such as older women (35). Preliminary work by

our team that included a systematic review of psychological interventions for older people exposed to trauma has identified a lack of tailored psychological interventions for older women affected by DA (29). Additionally, our Patient and Public Involvement (PPI) activities have consistently highlighted a significant gap in age-appropriate services and a pressing need for psychological support that recognises the unique experiences and barriers faced by older women.

Research questions

Main research question: Is it possible to adapt PATH in a way that is accessible to a diverse range of older women to empower and support their wellbeing and recovery?
1a) What does an adapted PATH intervention look like, and what types of formats and delivery are wanted?
1b) What are key stakeholders' views about current referral pathways?
1c) What are the barriers and facilitators for older women needing help?
1d) What solutions can be applied to optimise referral pathways for older women?
1e) Is it feasible to deliver A-PATH in primary care and community settings?
1f) What is the feasibility of collecting outcome measures and study procedures?
1g) What is the acceptability of A-PATH and trial conditions to older women and DA practitioners?
1h) How can the PATH intervention be adapted to effectively support the diverse needs of older women affected by DA including those from ethnic minority backgrounds and varied geographic settings?
1i) How can a revised fidelity framework ensure consistent and culturally responsive delivery across different contexts?

Methodology/Plan

The programme will be delivered over 36 months and will comprise 3 interlinked work streams (WSs). We will adapt PATH with the support and input of a Lived Experience Advisory Panel (LEAP). PPI will be embedded throughout all stages of the research, including through the LEAP panel. To ensure diversity and inclusivity, we will proactively recruit LEAP and PPI members through outreach specialist services that support older women from a range of ethnic, cultural and sexual identity backgrounds. One of our co-applicants brings lived experience of DA and will play a key role in project management, leading on PPI recruitment and co-facilitating the LEAP panel.

Design

We will use a mixed-methods design, a pilot randomised controlled trial incorporating acceptability and feasibility, and a process evaluation with embedded qualitative work (41) to:

a) WS1 - adapt the components and materials of PATH including practitioner training resources, and handouts for service users
b) WS2 - identify suitable and accessible referral pathways to facilitate older women's access to psychological support and advocacy services, and
c) WS3 - pilot test the feasibility and acceptability of A-PATH using a RCT design, alongside a nested qualitative sub-study to explore the acceptability,

experiences, and contextual factors influencing engagement with the intervention.

Work Stream 1

Part 1. *Adaptation and development of A-PATH*

In two focus group meetings (each with up to 8 survivors purposefully recruited to reflect diversity in age and ethnicity), we will explore the content of the intervention, elicit preferences for key topics (e.g., *Making sense of it all, Flashbacks and intrusive memories, Injured self-esteem*), and identify any gaps relevant to the needs of older women. The focus groups will be used to explore the current content and delivery of the PATH intervention, with the aim of understanding why it may not fully address the specific needs of older women (42). Discussions will identify specific elements of the intervention in relation to older women's experiences, as well as their preferences and barriers to accessing support. Prior qualitative findings and relevant literature (29) will be used to inform and enrich these discussions, ensuring they are grounded in existing evidence and lived experience.

While the PATH intervention has primarily been delivered to women who have left abusive relationships (30), we recognise that many older women remain in an abusive relationship due to complex factors such as dependency, care needs or housing barriers (18, 19). As part of this study, we will explore whether PATH is usable and acceptable for older women who are still in abusive relationships, and identify what adaptations may be necessary to ensure it is safe, appropriate, and responsive to their specific needs and circumstances. We will actively involve older women with lived experience in informing and shaping the adaptation of PATH in a meaningful and respectful way (42). When developing the intervention materials, we will draw on the linguistic and cultural expertise of our community partners to ensure accessibility and relevance for women whose first language is not English (43).

Our focus groups will be supported by our LEAP experts (44), and will invite older women survivors of DA to suggest improvements to PATH. The revised intervention will then be presented in a roundtable discussion with key stakeholders (representatives from Hourglass, Women's Aid, Age UK, DA practitioners; n = 8) to gather further feedback (35). This final stage will inform refinements to the content, materials, and logic model (see page 19). All components of the A-PATH intervention will be summarised using the Template for Intervention Development and Replication (TIDieR) checklist to support transparency and future scalability (45).

Preparation for the feasibility RCT

We will field-test our intervention with 4 survivors, to explore the practicalities of different delivery modes - telephone, online, and in-person - allowing us to identify any unaddressed barriers. In partnership with Solace Women's Aid (22), we will explore strategies to mitigate digital exclusion, which remains a significant barrier for older women. This could include for example offering non-digital options (e.g. phone, face-to-face, printed materials), providing basic digital skills support where appropriate, and working with community partners to ensure access to trusted spaces with digital resources. These strategies will be co-informed by the lived experience of older women to ensure accessibility, safety, and choice. We will also

explore practical factors influencing session attendance, such as mobility limitations, transport barriers and caring responsibilities (46).

Part 2. *Adaptation of training package for DA practitioners*

We will conduct two focus groups with DA practitioners (n=10), including those with and without experience working with older women, to assess the original PATH training package (30). To ensure the adapted PATH intervention is responsive to the needs of survivors from diverse ethnic, cultural, socioeconomic, and sexual identity backgrounds, we will also examine the inclusivity of the practitioner workforce, considering how diversity among practitioners can support more equitable and effective delivery. This includes exploring who delivers the intervention, how they are trained, and whether a more inclusive and representative workforce could enhance engagement with A-PATH. We will consider how practitioner background, cultural competence, and life experience influence delivery, and identify opportunities to strengthen training to support a more diverse, trauma-informed and inclusive approach.

Based on this input, we will develop and deliver a flexible training programme using a combination of online and face to face sessions, incorporating interactive and experiential learning methods. Each practitioner will receive a comprehensive training manual (47) that outlines session formats – including focus setting, targeted topic discussions, and progress evaluation (48) – alongside concrete examples to support delivery. These examples will guide practice in addressing common presenting problems identified among older women (49). To support skills development, we will provide one-to-one booster sessions, reflecting practitioner feedback and evidence that such sessions enhance the effectiveness of DA training (15, 49) by strengthening clinical skills (50). Group supervision will be provided throughout the delivery of A-PATH, using a hybrid model to accommodate online and in-person participation. Data from both sets of focus groups (Part 1 and Part 2) will be analysed using framework analysis (51) to identify key adaptations needed to ensure the intervention is appropriately tailored to the specific needs of older women.

Work Stream 2

Identifying suitable referral pathways

This Work Stream aims to improve access to psychological support for older women affected by DA by developing clear, setting-specific referral pathways across general practice, emergency departments, acute hospital wards, and community-based services (52). These settings are often the first point of contact for older women, where disclosures may occur in the context of experienced physical or mental health concerns. However, responses can be inconsistent, and older women's specific needs are frequently overlooked. Creating clear, setting-appropriate referral pathways is critical to ensuring timely, safe and supportive access to psychological interventions like A-PATH. This approach can help address existing service gaps and reduce missed opportunities for intervention. Informed by early stakeholder engagement, this will likely include the co-development of a referral protocol outlining clear referral criteria, practitioner guidance and structured signposting (53). To further inform this work, we will conduct individual interviews with survivors (n=10) and shadow helpline phone calls (n=10) in collaboration with our partner Hourglass. These will focus on identifying the structural and practical

challenges older women face when accessing support, and on developing strategies to help address and overcome them.

We will conduct two focus groups involving a diverse range of stakeholders, including GPs, nurses, psychiatrists, clinical psychologists, social workers, DA practitioners, third sector representatives, and survivors (n = 7- 8 participants per group; total n = 15), to explore: 1) the suitability of current referral pathways, with a focus on identifying barriers and facilitators that affect access to support for older women, and 2) the development of new referral pathways including practical guidance for use in specific settings. We will ask professionals and survivors how A-PATH can fit into existing services and how collaboration between primary care and community-based organisations can be strengthened (54). Prior to our feasibility trial we will examine referral routes into A-PATH to ensure they are accessible.

Data will be analysed thematically using both deductive approaches – based on pre-specified research questions – and inductive analysis to capture insights emerging from participants' accounts (51, 55). Emerging themes will be reviewed and validated by our LEAP experts. To support meaningful participation and relationship-building, we will allocate time early in the project to develop trust through community engagement strategies. This will include working with health ambassadors and community research champions who can act as trusted intermediaries, helping to foster relationships, and ensure the research is accessible, and inclusive (56, 57).

Work Stream 3

Feasibility and acceptability study of A-PATH

We will pilot A-PATH using an individually RCT design (58), in which eligible older women will be randomly assigned to receive either the adapted PATH intervention (plus DA advocacy) or usual care (DA advocacy only). We will assess the suitability of outcome measures, identify an appropriate primary outcome, and explore the acceptability of randomisation for older women affected by DA.

The primary aim is to assess whether a fully powered large-scale study is feasible and acceptable, by collecting information about the recruitment and trial retention rates that may affect the main trial. We will establish recruitment pathways, and identify facilitators and barriers to recruitment, including whether specific formats of the intervention (e.g. online or materials written in English) present a barrier to older women. Estimation of the (study) retention rate will enable us to assess whether a main trial is feasible, and to adjust for withdrawal appropriately. We will evaluate the feasibility of collecting outcome data, and explore participant burden, length of time and difficulty in completing measures and missing data. We will consider how best to balance and stratify our randomisation (e.g., by age, co-habitation status, and site), with our sites chosen to increase diversity and serve both rural and urban areas.

Target population and inclusion/exclusion criteria

We will recruit women aged 60 years or older who have experienced or continue to experience DA. Our study will focus specifically on intimate partner violence in later life, rather than broader forms of DA such as abuse by adult children or other family members. Our inclusion criteria will be as follows:

- women aged 60 years or older

- who have experienced domestic abuse, including physical, emotional, psychological, sexual, or financial abuse, either currently or in the past
- are able to provide informed consent to participate in the study
- have sufficient cognitive capacity to engage in the intervention and complete study assessments.

Although we will try to be as inclusive as possible, our PPI pre-grant advisory panel have advised us that we will need to exclude women with a psychotic illness, those living with a severe drug or alcohol problem, or those living with dementia. Individuals currently engaged in other structured psychotherapeutic treatments will also be excluded. We will only recruit older women, given evidence that they are more likely to experience repeated and severe forms of abuse compared to older male victims (59). We will purposively recruit older survivors from linguistically and culturally diverse backgrounds (e.g., South and West London), with participating services drawn from varied geographical areas across England and Wales. These sites reflect differing levels of prior engagement with older survivors, enhancing the diversity of perspectives and increasing the generalisability of our findings. For the purposes of adapting PATH, we define 'older women' as those aged 60 and above—a threshold that reflects the distinct social, psychological, and health-related experiences associated with later life, and aligns with best practice in DA advocacy for older survivors (18, 19).

Sample size calculation

As this is a feasibility study, the primary aim is not to detect a statistically significant effect but to estimate key parameters necessary for designing a future large scale trial, including acceptability of the intervention, and sensitivity to change of outcome measures (60, 61). Therefore, a formal power calculation is not appropriate at this stage. We will randomly allocate thirty-eight survivors to DA advocacy (usual care), and thirty-eight to DA advocacy plus A-PATH (n = 76). Our proposed sample size is based on recommended guidance for feasibility trials (58, 61), allowing sufficient precision to estimate feasibility outcomes, and gather preliminary data on outcome measures (e.g., mental health, wellbeing, service use). Findings from this feasibility trial will inform robust sample size calculations for a future large-scale RCT by providing estimates of effect size variability and other key design trial parameters.

The sample size of 76 that we have chosen allows us to estimate a retention rate of 75% at 8 months to within a 95% confidence interval of ±10%. This sample size will also enable us to estimate the standard deviation of our candidate primary outcome (Clinical Outcomes in Routine Evaluation–Outcome Measure (CORE-OM); psychological distress; (62, 63)) in our target population with sufficient precision (64). Based on existing data, we expect the standard deviation of CORE-OM to be approximately 5 (63), which we will estimate within a 95% confidence interval of ±0.9. The standard deviation estimate will inform the sample size calculation for our future larger trial. These sample size calculations were performed in Stata v18. These retention rates align with those observed in previous trials identified in our review, demonstrating their feasibility and achievability (29, 65). Outcome measures will be administered at baseline (prior to randomisation), at 4, 8 (primary outcome

timepoint), and 12 months post-randomisation to assess changes over time and inform follow-up feasibility.

Randomisation will be conducted by an unblinded member of the research team using a secure online randomisation system (64). In cases where a practitioner or researcher identifies a significant risk of self-harm, suicide, or harm from others, unblinding may be required to ensure appropriate and timely support for the participant's safety and well-being. To ensure the safety of both participants and the research team, we will develop and implement comprehensive risk management protocols, which we will adapt from our previous and ongoing studies (e.g. REPROVIDE (66)). Two distinct protocols will be established: one to guide responses to participant distress during or after study activities, and another to address safeguarding concerns related to ongoing abuse or risk of harm. These protocols will be informed by best practice guidelines and developed in consultation with our clinical (CH) and safeguarding (AW) leads to ensure a trauma-informed and ethically robust approach throughout the study.

Primary outcomes

Feasibility of progressing to a multi-centre RCT will be assessed primarily through recruitment and retention of participants, as well as acceptability and absence of any major concerns such as multiple serious adverse events. Satisfaction on these key points will indicate the practicality of scaling up the intervention.

Primary feasibility outcomes:

We will collect data on: a) the number of referrals and proportion of women recruited and randomised to the study out of those referred, b) the proportion of those retained at 4, 8, and 12 months, and c) the extent to which participants engage with the adapted PATH intervention as intended. Quantitative metrics which will include the percentage retained at primary endpoint (8 months) will be analysed alongside qualitative insights gathered through interviews with participants (both older women and practitioners) who withdraw or are lost to follow-up. This mixed-methods approach will enable us to assess the feasibility of achieving adequate retention and inform appropriate strategies to optimise it in a future full-scale RCT. Our stop/go progression criteria for retention (60) (baseline to primary outcome completion at 8-month follow-up out of number randomised) will be as follows:

a) trial retention rate approaching 80% will be considered supportive of feasibility for progression to a full trial
b) trial retention between 65% - < 80% indicative that a full trial is feasible, provided reasons for poor retention are identified and addressed, and
c) trial retention rates below 65% as indicative that our feasibility study is unlikely to progress to a full trial.

Intervention adherence will be assessed through session attendance, completion of core intervention components, and insights from qualitative follow-up interviews. We will explore reasons for non-attendance, with particular attention to barriers experienced by older women, to inform adaptations to the PATH intervention that enhance accessibility, engagement, and relevance for this population. As part of tailoring the PATH intervention to meet the specific needs of older women, we will adapt an existing fidelity framework to ensure core therapeutic components are delivered consistently while allowing appropriate flexibility for individual needs and

context (30, 31). This adapted framework will define essential elements of the intervention, identify adaptations needed (e.g. delivery mode, pacing, language), and include mechanisms for monitoring fidelity through practitioner training, supervision, and session checklists. This approach will help maintain intervention integrity while supporting relevance and responsiveness to older women's diverse experiences and preferences (35).

Secondary outcomes

As part of our feasibility RCT we will try to identify the most appropriate primary outcome measure for our large-scale multi-centre evaluation trial. We will estimate potential effectiveness on a range of candidate primary outcome measures, and their standard deviations (SDs). These will include:

a) psychological distress - Clinical Outcomes in Routine Evaluation–Outcome Measure (CORE-OM) (62); The 34-item CORE-OM assesses global psychological distress and is validated for sensitivity to change, test–retest reliability and UK normative comparison. Originally developed to evaluate the effectiveness of psychological therapies from several disciplines (67), it will be used in this study both as a continuous outcome (mean score) and a binary variable, using the clinical cut-off score of ≥ 9.9 (62).

b) depressive symptoms - Patient Health Questionnaire (PHQ) (68); The nine-item PHQ assesses depressive symptoms and demonstrates strong validity, reliability and sensitivity to change in older adults (69). In this study, it will be used as both a continuous measure (mean score) and a binary outcome based on the established clinical cut-off score of ≥10, indicative of probable major depression (69).

c) post-traumatic stress symptoms – Posttraumatic Stress Disorder Symptoms Scale (70); this 17-item questionnaire measures post-traumatic stress symptoms with a score ≥ 17 consistent with a clinical diagnosis of PTSD and has established validity in older samples (71), and

d) health-related quality of life - EQ-5D-5L (72); the EQ-5D-5L is a widely validated, standardised instrument that captures core dimensions of health, making it suitable for use across a range of populations and health conditions.

A ‘shortlist’ of potential primary outcome measures will be developed working closely with our LEAP group. We will explore participants’ and practitioners’ views of their suitability in the qualitative follow-up interviews. Participants will also be asked to record key demographic data at baseline and complete a brief feedback form when they finish the A-PATH sessions.

We consulted with lived experience experts to help us determine our primary outcome timepoint. Their feedback emphasised the importance of allowing enough time to observe meaningful and sustainable change. This input supported the selection of the 8-month timepoint as our primary outcome timepoint, ensuring participants have ample opportunity to complete A-PATH and to apply its content in their daily lives. This was considered especially important given the complexity of DA in later life. An 8-month follow-up allows us to capture longer-term indicators of impact and is supported by other DA intervention RCTs (15, 25, 30), and NIHR feasibility study guidance (41). We will also collect data at 12 months to assess sustained engagement and outcomes, with this timepoint serving as the primary outcome measure in the full-scale trial, consistent with our PROVIDE trial (30, 31).

Health-economic analysis

We will explore feasibility of collecting economic data, and establish the optimum way of evaluating cost-effectiveness in a multi-centre evaluation trial. Our health economic evaluation will consider the perspectives of health and social care, public sector and society, using Quality-Adjusted Life Years derived from the EQ-5D (72) and a modified Client Service Receipt Inventory (73). We will tailor our resource use questionnaire, in consultation with our PPI group and refine it through early testing. We will explore the completeness, acceptability, and relevance of these measures, including the feasibility of collecting data from both NHS and wider service perspectives.

Process evaluation

We will use realist methodology to identify what works, for whom and in what circumstances (74), utilising the Medical Research Council (MRC) process evaluation framework (75). A key focus of the process evaluation will be to explore the acceptability of the A-PATH intervention, including how it is experienced by participants and practitioners within real-world settings. We will conduct qualitative process interviews with up to 15 survivors who participated in the A-PATH intervention, capturing a range of engagement levels, alongside interviews with up to 10 survivors from the usual care group to explore comparative experiences. Where possible, we will also conduct short informal 'exit' interviews (participants who drop out of the intervention or study), to better understand reasons for disengagement. We will interview DA practitioners (n = 6) to gather insights into what elements of the intervention are working well, and what needs to improve, and to inform our understanding of A-PATH's mechanisms of change (31).

The process evaluation work will enable us to identify candidate factors that may require adaptation within the PATH protocol, thereby informing refinements aimed at improving retention among older women in a future large trial. We will explore DA practitioners' perspectives on factors that facilitate or hinder engagement among diverse groups of older women and how obstacles can be overcome (76). We will use this data to inform the refinement of our logic model to be fully tested in a future large-scale RCT (43). A comprehensive flow chart detailing all stages of the research is presented in page 20. This final Work Stream will synthesise the study's key outputs, including the adapted intervention, an updated logic model, a co-produced referral pathway, and the delivery of a feasibility pilot RCT.

Health technologies being assessed

A-PATH Intervention

Women randomised to A-PATH will be offered up to eight weekly or fortnightly sessions, followed by two booster sessions, 1 month and then 3 months post-intervention. The schedule will be flexible and revised according to participant preferences. A-PATH is designed to support women in recognising, and exploring psychological consequences related to experiences of DA, while developing coping strategies to manage conflict and other stressors (30, 31). In collaboration with survivors and practitioners we have identified several adapted topics relevant to older women, including "What I Want to Happen" and "Planning for the Future" which will be incorporated into the intervention content. The therapeutic sessions will be

delivered alongside standard DA advocacy support. When adapting PATH, we will draw on the '*psychosocial readiness mode*l' of change (77), which highlights awareness, perceived support and perceived power as key factors influencing behaviour change in women affected by DA. As part of this work, we will produce a revised logic model with specific behaviour change components for this group.

Recognising the diverse barriers faced by older women including digital exclusion, mobility limitations, frailty, and social isolation—the intervention will employ a flexible delivery model that accommodates individual needs and preferences. Participants will be offered flexible access to the intervention through multiple formats—face-to-face sessions in accessible community or healthcare settings, telephone-based therapy, or online video calls—ensuring it is both feasible and tailored to individual preferences. Insights from our lived experiences study indicated that this flexibility ensures that women without internet access or digital skills (e.g. those aged over 80; (78)), can still engage fully, while those comfortable with technology have the option of remote support. All DA practitioners will receive regular supervision by RAD (25), throughout the study period.

Usual care

Women in the control group will receive usual care in the form of DA advocacy support but will not participate in A-PATH sessions. The advocacy support will include a package of legal, housing, financial, and safety advice, provided by our partner Hourglass, a leading national charity and experts in DA of older women (8). This support will be provided by trained DA practitioners to ensure women receive accurate, timely, and trauma-informed guidance. The package is designed to empower survivors with critical knowledge about their rights, options for financial independence, and personalised safety strategies, and support women in making informed decisions about their future. Advocates in this group will not receive specialist training in psychological methods.

What might the A-PATH intervention look like*?

A-PATH is designed to be **flexible** and **client-led**. In the first session, the practitioner and client explore the psychological impact of abuse by focusing on **validating lived experiences.** Subsequent sessions are collaboratively selected by the client and practitioner, ensuring A-PATH remains responsive and person-centred. Core themes include **strengthening self-esteem**, **promoting self-care**, **reducing trauma and psychological distress**, and **building emotional resilience** to support recovery and wellbeing. The final session focuses on reviewing progress, reinforcing key strategies, and **planning for the future.**

** Informed by the original PATH trial, our qualitative research, lived experiences, and wider evidence on tailoring DA interventions for older women (15, 25)*

Knowledge mobilisation, dissemination and impact

Knowledge mobilisation

Despite progress in DA research and policy, older women survivors continue to be marginalised, and experience social, economic, emotional, and institutional exclusion (19). Our aim is to translate research into meaningful action by:

a) building an interactive A-PATH website, with survivors and practitioners, to share findings in accessible, engaging formats and actively invite feedback from a broad public and professional audience;
b) collaboratively developing a set of evidence-informed referral pathways for use across the NHS, local authorities, and the third sector;
c) facilitating co-designed workshops and roundtable discussions with survivors, frontline professionals, and community organisations to reflect on our findings and collaboratively shape the design of a future large-scale trial.

We will follow guidance on community-based knowledge mobilisation in DA research (79, 80). Central to our approach is the development of equitable partnerships with survivors, practitioners, and third-sector organisations, recognising their essential roles in shaping, delivering, and sustaining meaningful change.

Dissemination

Working with key stakeholders, and lived experience experts we will co-develop a dissemination plan to identify who we need to engage, how to reach them and ensure outputs are accessible and inclusive. We will draw on our National Peer Research Network for Preventing Domestic Abuse in Older People, hosted by UCL, which brings together survivors, researchers, practitioners, and community organisations across the UK. The network will help us reach a broad and diverse audience including health and social care professionals and third-sector organisations. Our dissemination activities will be facilitated with the support of our LEAP experts and third sector partners. We will publish our trial findings in peer-reviewed academic journals and present these in national DA conferences. We will attend local community events attended by commissioners of DA services to present our results and host a public knowledge exchange event to share our findings and our 'good practice' referral pathway.

Impact

This feasibility trial will provide critical preparatory work for addressing a major gap in supporting older women affected by DA. We anticipate direct impact on improving awareness of DA affecting older women, and guidance on suitable referral pathways including videos describing our key findings. By adapting the evidence-based PATH intervention to meet the specific needs of older women, this study will improve its future accessibility by tailoring its content to the unique experiences, health profiles and barriers faced by older women. This study will also strengthen the current evidence base by generating essential data on recruitment, retention, and outcome measurement in this population. Developing an intervention that is shaped by lived experience and is responsive to the psychological, social and practical needs of older women allows us to amplify the voices of older survivors, ensuring we lay the foundations for scalable change in DA support services and practice across health and third-sector settings. If A-PATH is feasible and acceptable, it will lead to a larger HTA trial evaluating the first evidence-based trauma-informed and age-responsive psychological intervention for older women affected by DA.

Project management

Timeline, and milestones of the project

The project will be delivered over 36 months. The first 9 months will focus on adapting the PATH intervention and co-developing setting-specific referral pathways. This will be followed by a 23-month feasibility study, including an 8-month recruitment phase and a 12-month outcome assessment period. In the final 6 months we will complete data analysis, interpretation, and dissemination through reports, academic publications, and stakeholder engagement.

Work Stream 0 (Months 3–0): We will obtain all necessary NHS Ethics and Health Research Authority (HRA) approvals between October and December 2025—three months before the planned study start date—to ensure full regulatory compliance and allow adequate time for site setup and preparatory activities.

Work Stream 1 (Months 1–4): We will conduct two participatory focus groups and a roundtable discussion with key stakeholders to inform the adaptation of the PATH intervention. During this phase, we will also revise the original training package for DA practitioners to ensure it is relevant for supporting older women. During this WS we will field-test the adapted PATH intervention to gather preliminary data on barriers and facilitators in real-world settings.

Work Stream 2 (Months 5–9): We will conduct individual interviews with survivors and focus groups with professionals to explore and optimise setting-specific referral protocols across services such as general practice, emergency care, acute wards, and community settings.

Work Stream 3 (Months 10–36): A 23-month feasibility study will be undertaken (Months 10–23), including a 8-month recruitment phase (Months 10–17), supported by our established National Network, third sector partners and participating NHS Trusts. Training for DA practitioners will take place before recruitment begins to ensure preparedness across sites. Quantitative and qualitative data will be analysed and disseminated through academic publications (Months 31-36), conference presentations and community stakeholder workshops.

Recruitment centres and partners

Building connections to support recruitment. Recruitment of older women affected by DA requires building trusted connections with services and communities that support this population. We will engage with NHS settings (e.g. general practice, and mental health services), third-sector organisations and community groups working with older adults. These partnerships will help us identify potential participants sensitively and ethically, while ensuring outreach is inclusive of women from diverse backgrounds, drawing upon established IRIS (Identification and Referral to Improve Safety) sites (34) and our existing networks. Our feasibility trial will be delivered across **four principal recruitment centres**—London, Bristol, Swansea, and North Devon—**each supported by two affiliated local recruitment sites (NHS and community-based),** all with established research infrastructure.

- **London (University College London; 4 sites; projected recruitment 32 participants)**: UCL brings extensive experience in leading complex mental health and DA trials. It has well-established NHS and community sector partnerships to support recruitment and delivery, and led our lived experiences study. This centre will recruit from the North West London NHS Foundation Trust, Camden and Islington NHS Trust (and partner Trusts through Noclor), and local centres (Age UK, North London Women's Aid and SafeLives), contributing a total of 4 individual sites.

- **Bristol (University of Bristol; 2 sites; projected recruitment 16 participants)**: University of Bristol is a national and international leader in DA research. The team at Bristol has longstanding collaborations with local services and a proven track record of delivering intervention studies with women affected by DA. The Bristol team will recruit from NHS Bristol, North Somerset and South Gloucestershire Trust, and local DA services that participate in ongoing trials (Bristol Next Link).
- **Swansea (Dewis Choice; 2 sites; projected recruitment 14 participants)**: Dewis Choice is the only UK initiative specifically designed for older survivors of DA, with expertise in age-informed, trauma-sensitive practice. This site will work closely with the Centre for Age, Gender and Social Justice, Solace Women's Aid and the Swansea Bay University Health Board.
- **North Devon (North Devon Against Domestic Abuse – NDADA; 2 sites; projected recruitment 14 participants)**: As a trusted local service with embedded links to health and social care providers, NDADA offers crucial insight into rural service delivery for older women affected by DA in underserved areas. This site will receive referrals directly from NDADA and the Royal Devon University Healthcare NHS Foundation Trust.

Our four recruitment centres provide geographic and demographic diversity, supported by strong community and clinical networks across ten active recruitment sites. Based on empirical benchmarks from NIHR feasibility studies (81, 82), we anticipate a recruitment rate of one participant per site per month. Across ten sites over 8 months—the recommended window for engaging underserved groups—this provides a potential total of 80 participants, allowing a buffer above our target of 76 to mitigate potential recruitment challenges. This estimate reflects realistic feasibility assumptions (61) and is enabled by our plans to include national recruitment pathways e.g. via Hourglass helplines and an online delivery option.

Third sector partners

To ensure inclusive recruitment, we will collaborate with key third sector organisations that have established expertise in supporting older women affected by DA. These include **Hourglass**, the only UK-wide charity dedicated to ending abuse and neglect of older people; **Women's Aid**, a national federation supporting women experiencing DA; **SafeLives**, a leading organisation with cross-sector frontline expertise; **Age UK**, which delivers safeguarding and support services for older adults; and **Dewis Choice**, a pioneering Welsh initiative co-produced with older survivors and led by one of our co-applicants. We will also recruit through **North Devon Against Domestic Abuse (NDADA**), a specialist provider of refuge and community-based support for women in rural and underserved areas. These trusted partners will support participant identification, DA practitioner recruitment, and help develop trauma-informed, age-sensitive engagement strategies. All materials will be co-designed with our LEAP group to ensure they are accessible, respectful, and reflective of older women's experiences. The longstanding credibility and community presence of our partners will be central to improving reach, trust, and engagement with underserved populations.

NHS Trust partners

Recruitment through NHS Trusts will be a key component of our feasibility trial. We will work closely with general practice teams including mental health services, community mental health older adult teams, safeguarding leads, and health-based independent domestic violence advisors (IDVAs). These professionals are well placed to identify older women who may be experiencing DA but are not currently accessing specialist support. Site-specific research champions from our National Network will promote the study among frontline staff.

Recruitment of DA Practitioners

We have already identified 1 practitioner (DA champion) in each of our sites. The remaining practitioners will be identified through established DA support networks, our partner Hourglass, specialist services, and community organisations and will be led by our safeguarding lead (AW) and our DA champions. We will adopt a proactive, evidence-based recruitment strategy aligned with our previous trial (30), and the landmark IRIS study (54), where each study site has a designated advocate coordinating referrals. Our community-led approach—centred on collaboration with survivors, frontline professionals, and voluntary sector leaders—places us in a strong position to deliver this research effectively. Community outreach will be a key component of our engagement strategy (83), supported by our National peer research network preventing domestic abuse in older people initiative.

Study management, and expertise in the study team

Our multidisciplinary team brings together expertise from psychology, social science, methodology, statistics, general practice, nursing, DA advocacy and lived experience. Collectively, we have a strong track record in the development and evaluation of complex, community-based interventions for older adults and women affected by DA. Our team includes Dr Vasiliki Orgeta, a specialist in psychological interventions in later life (29), who co-led the recent Lancet Commission on Trauma and Ageing, which highlighted the underrepresentation of older women affected by DA in both research and policy (84). Dr Helen Cramer is a nationally recognised leader in perpetrator-focused interventions (57, 66). They will receive institutional support from both University College London and the University of Bristol—centres of excellence in older adults' mental health and DA research.

Dr Roxane Agnew-Davies is a clinical psychologist with over 30 years of experience in specialist mental health services for women affected by DA. She led the development of PATH implemented in various sites across England and Wales and has published widely on DA. Dr Alison Gregory is a nationally recognised expert on the secondary trauma experienced by informal supporters of DA survivors. She has led multiple impactful research projects with strong partnerships with Women's Aid, local authorities, and public policy stakeholders to translate research into practice.

Dr Victoria Vickerstaff brings expertise in statistical methodology and applied health research, with a robust track record of leading the design and analysis of several multidisciplinary NIHR-funded trials in older people and palliative care settings. Amanda Warburton-Wynn is the Domestic Abuse Health Strategic Lead for Cambridgeshire County Council, working with health agencies across the county to improve the recognition and response to domestic abuse. She developed the Older People's DASH risk assessment tool for DA that has been used in Cambridgeshire

since 2021 and is currently being piloted in Norfolk and Wales. She is also an independent researcher, specialising in improving support for older people and people with disabilities who are subjected to DA and sexual violence.

Marion Goodchild brings over 25 years of experience in nursing with a strong track record of championing patient-centred approaches in health and social care research, such as enhanced training for cancer care teams in identifying DA. She has contributed to multiple research programmes as a public advisor, helping to improve participant engagement and inclusive practice. Dr Claire Hawcroft is a practising GP and clinical academic with expertise in the design and evaluation of trauma-informed interventions for women affected by DA across primary care and community settings. Professor Sarah Wydall is a leading expert on DA in later life. As Principal Investigator of the Dewis Choice project, she has pioneered multi-agency, participatory responses for older survivors, including those with dementia, and developed innovative tools and frameworks now used nationally.

Data management, reporting and analysis

Questionnaire and clinical data will be collected and stored securely in REDCap, with access restricted to authorised research team members. Administrative data—such as participant names, contact details, and follow-up information—will be pseudonymised and stored in a separate, password-protected database (e.g. Microsoft Access), in accordance with data protection regulations. This separation of identifiable and research data ensures confidentiality, minimises risk of data breaches, and complies with Good Clinical Practice (GCP). All personal data, and data handling, processing, storage, and destruction will comply with the Data Protection Act 2018 and GDPR regulations. Data quality assurance procedures will be in place to verify the accuracy of information entered from any hard-copy questionnaires. Recruitment and retention data will be summarised using a CONSORT flow diagram. A statistical and health economic analysis plan for the feasibility trial will be developed by the trial statistician (VV) in collaboration with the Trial Management Group.

Study oversight and safety protocol

Oversight will be provided by our LEAP panel, which will meet quarterly to inform and contribute to all Work Streams. The Programme Management Group (PMG) will meet monthly throughout the study to oversee delivery, with a particular focus on monitoring recruitment against projected targets. Meetings will include regular review of site-level performance, identification of emerging challenges, and implementation of corrective actions where necessary to ensure timely and effective recruitment. Additional ad hoc meetings will be convened should any significant issues arise.

The quality and integrity of trial data will be monitored by an independent Data Monitoring and Ethics Committee (DMEC) and a Trial Steering Committee (TSC). A comprehensive safety protocol, aligned with WHO guidelines (85), will be used to record and monitor adverse events. The TSC will review any reported harms associated with the A-PATH intervention. All serious adverse events will be reported immediately to the sponsor, TSC, and the NHS Research Ethics Committee (REC) in accordance with the UK Policy Framework for Health and Social Care Research (2020). Participant safety will be overseen by our designated Safeguarding Lead

(AW), who will conduct monthly safety reviews and report findings to both the PMG and LEAP.

If participants (older women or practitioners) become distressed during any sessions or assessments, the researcher/DA practitioner will respond in line with established safeguarding and distress protocols. Immediate emotional support will be offered, and the session paused or discontinued if needed. With the participant's consent, a referral will be made to appropriate support services, including local DA specialists and mental health providers. All research staff will be trained in trauma-informed approaches and will follow a clear distress and a separate safeguarding protocol approved by the ethics committee. Our study is informed by best practice guidelines from UK third sector organisations, including SafeLives, Women's Aid, and AVA (Against Violence and Abuse), emphasising the importance of conducting research that is trauma-informed, survivor-centred, and grounded in principles of safety and confidentiality. Researchers will be trained to recognise distress, and prioritise participant autonomy and well-being. All procedures will align with the ethical standards outlined by the UK Research Integrity Office (UKRIO) and the HRA guidance for research involving vulnerable adults, with input from our lead specialist DA advisor (AW) during our protocol development.

Equality, diversity, and inclusion

We have considerable expertise in recruiting participants from diverse communities. We will engage with seldom heard populations (e.g., older women from Black, Asian and minoritised ethnic communities, those that identify as LGBTQIA+), which face unique barriers in coping with abuse (76, 86). To encourage recruitment from non-English speaking communities, we will implement inclusive and culturally sensitive engagement strategies throughout the study. We will work with established leaders (e.g., Hourglass, Women's Aid), to ensure wording, images, and recruitment methods are as inclusive as possible. Recruitment materials (i.e., participant information sheets, posters, briefings to teams) will be reviewed for cultural appropriateness. We will work in partnership with community organisations that have established trust within ethnic minority communities to raise awareness of the study. We will work closely with local services and faith groups that support minoritised communities, to increase our insight of how best to serve these communities. We will collect data on individuals who were potentially eligible but excluded due to limited English proficiency, to inform inclusive recruitment strategies for a full-scale RCT, ultimately improving the generalisability of the adapted PATH intervention.

Intellectual property and commercialisation

All novel materials developed during this project will be identified and documented as potential intellectual property (IP). We will work closely with UCL's IP team to develop our IP strategy, and ensure that any outputs maximise public benefit while maintaining accessibility for organisations who support older women affected by DA. The overarching aim is to ensure that A-PATH delivers lasting value — improving outcomes for older survivors, and equips third-sector organisations with the tools to support their recovery and well-being. We will explore opportunities to expand the programme's reach through partnerships with third-sector organisations, licensing arrangements, or social enterprise models.

Logic model of A-PATH - Adapting Psychological Advocacy Towards Healing for older women affected by domestic abuse

Population

Population:
Women aged 60 years or more

Physical health risks
- Increased risk of frailty
- Increased incidence of chronic conditions

Psychological harm
- Depression, anxiety and trauma symptoms
- Shame & self-blame

Social risks
- Isolation
- Stigma
- Lack of support

Financial harm
- Economic abuse

Systemic risks
- Ageism in services
- Limited access to support

Theory, Assumptions & Implications

Empowerment Theory
- Building autonomy
- Enhancing self-efficacy
- Challenging systemic barriers

Assumptions & implications
- Gaining control
- Building confidence
- Promoting healthy relationships
- Empowerment through access to knowledge
- Autonomy and voice

Approach
- Focusing on strengths
- Avoiding paternalism
- Clear & accessible information
- Challenging ageism, stigma and exclusion

Intervention

A-PATH intervention
- Uses the psychosocial readiness model & CBT principles
- Strategies to cope with distress
- Enhances self-esteem
- Strengthens resilience
- Promotes self-care

Mechanisms of change
- Awareness
- Perceived support
- Perceived power

Control group
- Receives usual DA advocacy

DA advocacy
- Practical, legal, financial & safety advice
- Delivered by third sector and local DA services

Short Term outcomes

Psychological well-being:
- Improved self-esteem
- Reduced psychological distress

Sense of control
- Increased confidence
- Increased self-efficacy
- Greater control over routines & choices

Social connection
- Reduced social isolation
- Improved trust in others
- Rebuilding relationships

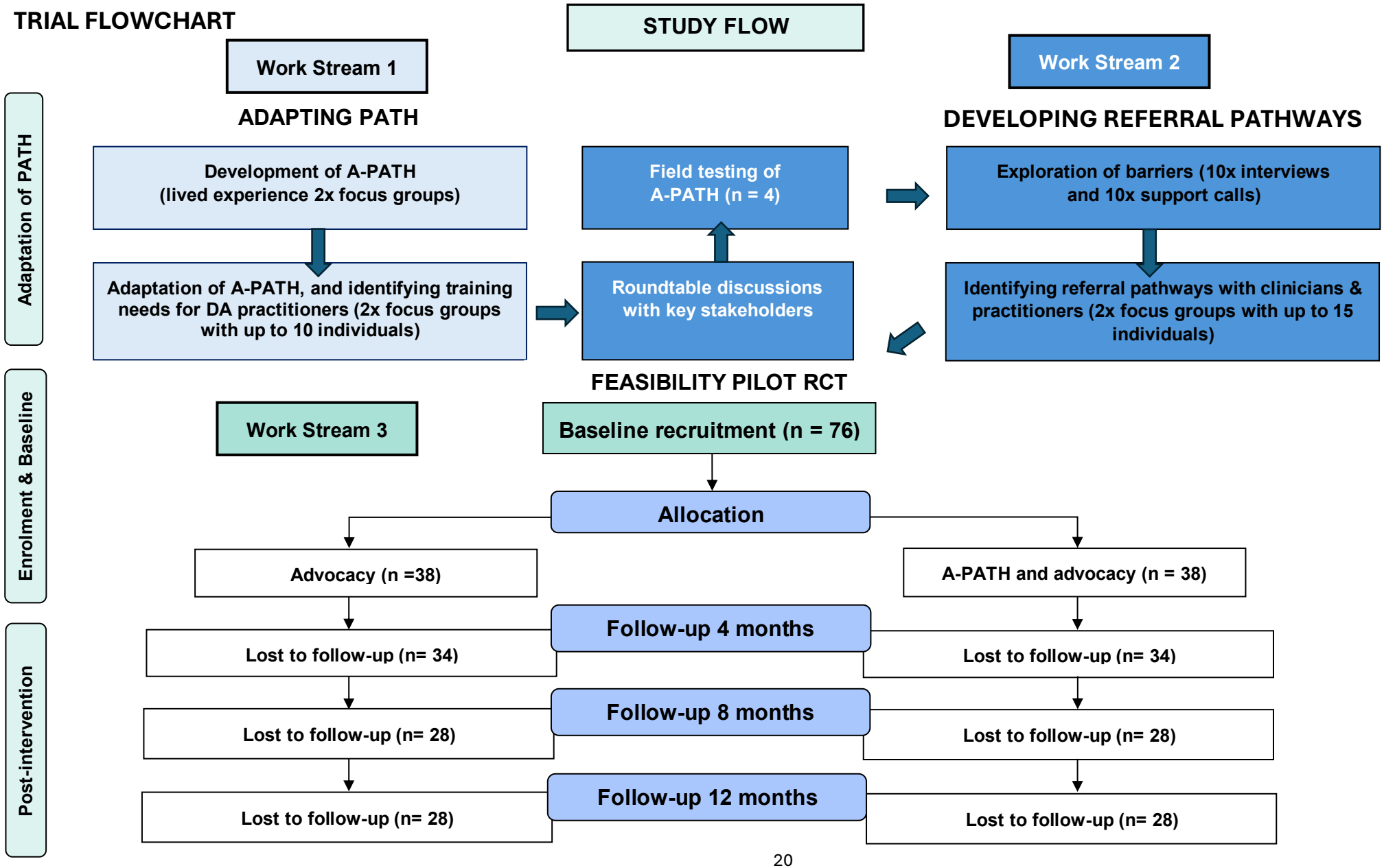
Long Term Outcomes

Psychological well-being:
- Reduce depression
- Reduce trauma symptoms
- Living with dignity
- Thriving beyond safety
- Improved well-being
- Strengthening resilience

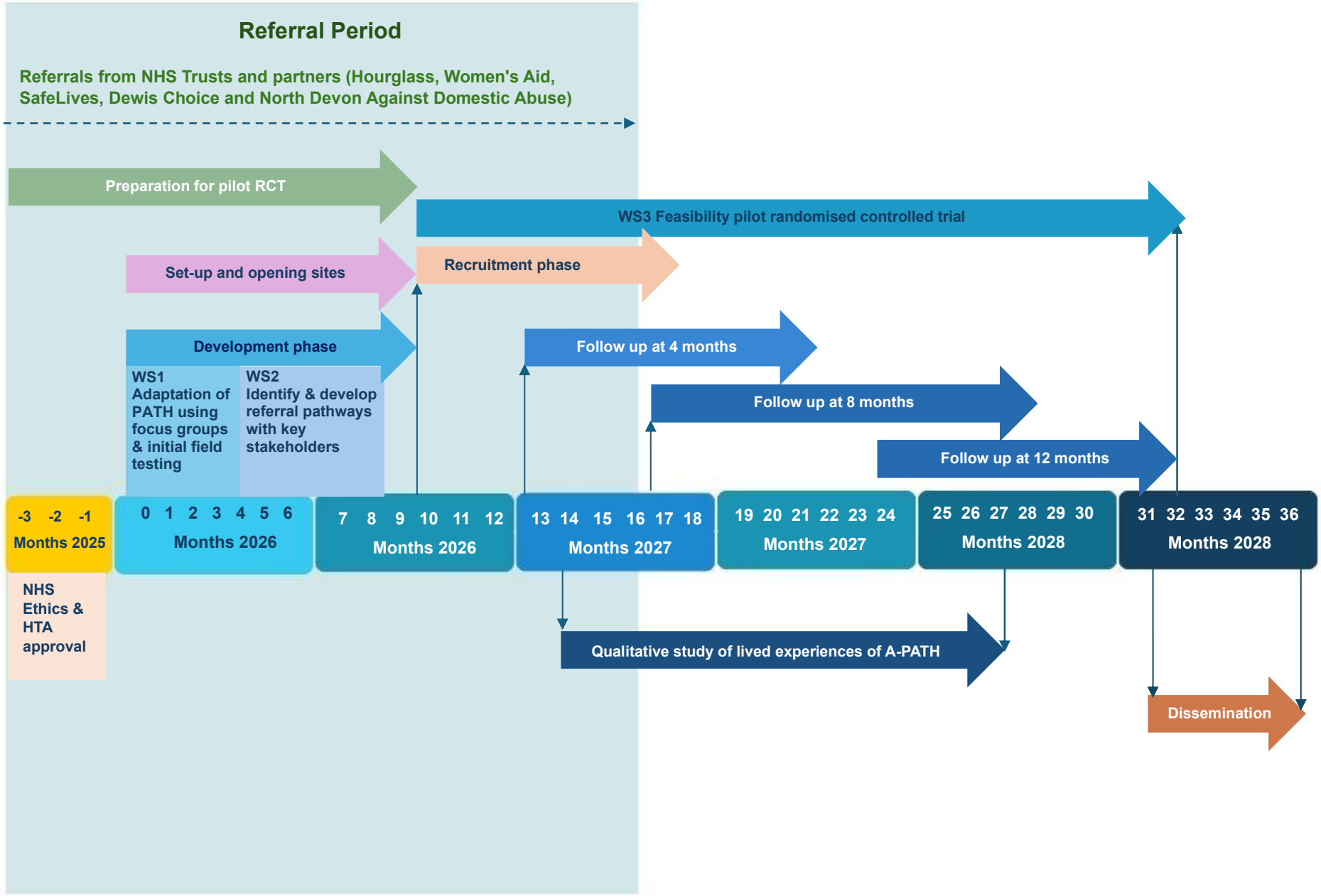
Safety
- Awareness of risk
- Reduction in exposure to harm

Services engagement
- Increased awareness of services
- Improving ability to navigate services

Structural change
- Inclusive care systems
- Tackling ageism and stigma


TRIAL FLOWCHART
STUDY FLOW
Work Stream 1
Work Stream 2
ADAPTING PATH
DEVELOPING REFERRAL PATHWAYS
Adaptation of PATH
Development of A-PATH
(lived experience 2x focus groups)
Field testing of
A-PATH (n = 4)
Exploration of barriers (10x interviews
and 10x support calls)
Adaptation of A-PATH, and identifying training
needs for DA practitioners (2x focus groups
with up to 10 individuals)
Roundtable discussions
with key stakeholders
Identifying referral pathways with clinicians &
practitioners (2x focus groups with up to 15
individuals)
FEASIBILITY PILOT RCT
Enrolment & Baseline
Work Stream 3
Baseline recruitment (n = 76)
Allocation
Advocacy (n =38)
A-PATH and advocacy (n = 38)
Post-intervention
Follow-up 4 months
Lost to follow-up (n= 34)
Lost to follow-up (n= 34)
Follow-up 8 months
Lost to follow-up (n= 28)
Lost to follow-up (n= 28)
Follow-up 12 months
Lost to follow-up (n= 28)
Lost to follow-up (n= 28)

Time Plan for the A-PATH Project



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